

Bates' Guide to Physical Examination and History Taking

Fourteenth Edition



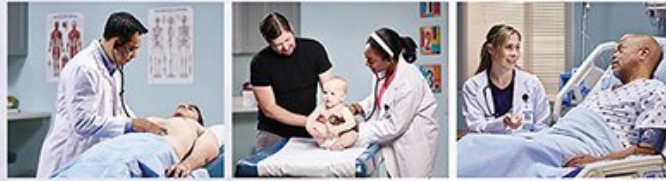
Rainier P. Soriano



Wolters Kluwer

Bates' Guide to Physical Examination and History Taking

Fourteenth Edition



Rainier P. Soriano

 Wolters Kluwer

Bates' Guide to Physical Examination and History Taking

Fourteenth Edition

Lead Author and Editor

Rainier P. Soriano, MD

Professor of Medical Education, Geriatrics and Palliative Medicine
Brookdale Department of Geriatrics and Palliative Medicine
Senior Associate Dean for Curricular Affairs
Leni and Peter W. May Department of Medical Education
Icahn School of Medicine at Mount Sinai
New York, New York

Co-Editors

Peter G. Szilagyi, MD, MPH

Distinguished Professor of Pediatrics
David Geffen School of Medicine at UCLA
Los Angeles, California

Richard Hoffman, MD, MPH, FACP

Emeritus Professor
Department of Medicine
University of Iowa Carver College of Medicine
Iowa City, Iowa

Editor Emeritus

Lynn S. Bickley, MD, FACP

Clinical Professor of Internal Medicine
University of New Mexico School of Medicine
Albuquerque, New Mexico



Philadelphia • Baltimore • New York • London
Buenos Aires • Hong Kong • Sydney • Tokyo

Acquisitions Editor: Matt Hauber
Development Editor: Deborah Bordeaux
Senior Editorial Coordinator: Sean Hanrahan
Editorial Assistant: Parisa Saranj
Marketing Manager: Kirsten Watrud
Senior Production Project Manager: Catherine Ott
Manager, Graphic Arts & Design: Stephen Druding
Art Director, Illustration: Jennifer Clements
Illustrator: Body Scientific International
Photographer: Mike Stog at DesignWorksVisualMedia
Media Production: Liz Simmons Thibodeau at Thibodeau Media Group
Manufacturing Coordinator: Margie Orzech
Prepress Vendor: Aptara, Inc.

Fourteenth edition

Copyright © 2026 Wolters Kluwer.

Copyright © 2017 Wolters Kluwer. Copyright © 2013, 2009 by Wolters Kluwer Health/Lippincott Williams & Wilkins. Copyright © 2007, 2003, 1999 by Lippincott Williams & Wilkins. Copyright © 1995, 1991, 1987, 1983, 1979, 1974 by J. B. Lippincott Company. All rights reserved. This book is protected by copyright. No part of this book may be reproduced or transmitted in any form or by any means, including as photocopies or scanned-in or other electronic copies, or utilized by any information storage and retrieval system without written permission from the copyright owner, except for brief quotations embodied in critical articles and reviews. Materials appearing in this book prepared by individuals as part of their official duties as U.S. government employees are not covered by the above-mentioned copyright. To request permission, please contact Wolters Kluwer at Two Commerce Square, 2001 Market Street, Philadelphia, PA 19103, via email at permissions@lww.com, or via our website at shop.lww.com (products and services).

9 8 7 6 5 4 3 2 1

Printed in Mexico

Library of Congress Cataloging-in-Publication Data

Names: Soriano, Rainier P., author, editor. | Szilagyi, Peter G., editor. | Hoffman, Richard M., editor. | Bickley, Lynn S., editor. | Bickley, Lynn S. Bates' guide to physical examination and history taking.

Title: Bates' guide to physical examination and history taking / lead author and editor, Rainier P. Soriano; Peter G. Szilagyi, Richard Hoffman, Lynn S. Bickley, [editors].

Other titles: Guide to physical examination and history taking

Description: Fourteenth edition. | Philadelphia, PA: Wolters Kluwer, [2026] | Preceded by Bates' guide to physical examination and history taking / Lynn S. Bickley, Peter G. Szilagyi, Richard M. Hoffman, Rainier P. Soriano. [2024] | Includes bibliographical references and index. | Summary: "Bates' Guide to Physical Examination and History Taking, 14th Edition provides complete and authoritative guidance on performing physical examinations successfully. The focus is on training students on the skills needed for patient encounters - head-to-toe and physical examination skills, history taking, communication, patient note taking, and often touching on clinical reasoning and differential diagnosis"—Provided by publisher.

Identifiers: LCCN 2024055552 (print) | LCCN 2024055553 (ebook) | ISBN 9781975218348 (paperback) | ISBN 9781975218362 (epub)

Subjects: MESH: Physical Examination—methods | Medical History Taking—methods

Classification: LCC RC76 (print) | LCC RC76 (ebook) | NLM WB 205 | DDC 616.07/54—dc23/eng/20250204

LC record available at <https://lcn.loc.gov/2024055552>

LC ebook record available at <https://lcn.loc.gov/2024055553>

This work is provided “as is,” and the publisher disclaims any and all warranties, express or implied, including any warranties as to accuracy, comprehensiveness, or currency of the content of this work.

This work is no substitute for individual patient assessment based upon healthcare professionals' examination of each patient and consideration of, among other things, age, weight, gender, current or prior medical conditions, medication history, laboratory data and other factors unique to the patient. The publisher does not provide medical advice or guidance and this work is merely a reference tool. Healthcare professionals, and not the publisher, are solely responsible for the use of this work including all medical judgments and for any resulting diagnosis and treatments.

Given continuous, rapid advances in medical science and health information, independent professional verification of medical diagnoses, indications, appropriate pharmaceutical selections and dosages, and treatment options should be made and healthcare professionals should consult a variety of sources. When prescribing medication, healthcare professionals are advised to consult the product information sheet (the manufacturer's package insert) accompanying each drug to verify, among other things, conditions of use, warnings and side effects and identify any changes in dosage schedule or contraindications, particularly if the medication to be administered is new, infrequently used or has a narrow therapeutic range. To the maximum extent permitted under applicable law, no responsibility is assumed by the publisher for any injury and/or damage to persons or property, as a matter of products liability, negligence law or otherwise, or from any reference to or use by any person of this work.

shop.lww.com

To Lynn Bickley, whose dedication to the art and science of physical diagnosis has inspired generations of clinicians and laid the foundation for countless students; it is an honor to continue your legacy.

Faculty Contributors

GEORGE A. ALBA, MD

Assistant Professor of Medicine
Division of Pulmonary and Critical Care Medicine
Massachusetts General Hospital
Harvard Medical School
Boston, Massachusetts

CARA A. BROWN, MD, AEMUS-FPD, FACEP

Assistant Professor
Director, Advanced Emergency Ultrasound Fellowship
Department of Emergency Medicine
Icahn School of Medicine at Mount Sinai
New York, New York

CHRISTOPHER T. DOUGHTY, MD

Assistant Professor of Neurology
Harvard Medical School
Brigham and Women's Hospital
Boston, Massachusetts

NICOLA A. FELDMAN, MD, MS

Resident Physician
Boston Combined Residency Program in Pediatrics
Boston Children's Hospital and Boston Medical Center
Boston, Massachusetts

ROCCO M. FERRANDINO, MD, MSCR

Assistant Professor
Department of Otolaryngology-Head and Neck Surgery
University of Washington
Seattle, Washington

RAISA GAO, MD, FACOG

Vice Chair
Department of Obstetrics and Gynecology
Huntington Hospital
Northwell Health
Assistant Professor
Donald and Barbara Zucker School of Medicine at Hofstra
Hempstead, New York

SARAH M. GUSTAFSON, MD

Associate Clinical Professor
Division of Pediatric Hospital Medicine
David Geffen School of Medicine at UCLA
Los Angeles, California

MICHAEL L. HERSCHER, MD

Associate Professor of Medicine and Medical Education
Division of Hospital Medicine
Mount Sinai Hospital
Icahn School of Medicine at Mount Sinai
New York, New York

ALEXANDER R. LLOYD, MD

PM&R Sports Medicine Physician
Assistant Program Director, Sports Medicine Fellowship
Rehabilitation and Performance Medicine
Providence Swedish
Seattle, Washington

CHRISTOPHER C. LO, MD

Orbital and Oculofacial Plastic Surgery
Medical Director
Eyesthetica
Los Angeles, California

STEPHEN A. McCULLOUGH, MD

Assistant Professor of Clinical Medicine
Division of Cardiology
Weill Cornell Medicine
New York Presbyterian Hospital
New York, New York

BRET P. NELSON, MD, AEMUS-FPD, FACEP

Professor, Department of Emergency Medicine
System Vice Chair, Education
Chief, Emergency Ultrasound Division
Department of Emergency Medicine
Icahn School of Medicine at Mount Sinai
New York, New York

MATTHEW E. POLLARD, MD

Reproductive Urology
Regional Medical Director, Vice President of Quality and Clinical Excellence
Posterity Health, PC
Nashville, Tennessee

BESS M. STORCH, MD

Assistant Professor

Department of Emergency Medicine
Mount Sinai West-Morningside Hospitals
Department of Medical Education
Icahn School of Medicine at Mount Sinai
New York, New York

Preface

I am excited to introduce the fourteenth edition of *Bates' Guide to Physical Examination and History Taking*, a trusted resource that has guided generations of health care learners in developing the essential skills of patient assessment. This edition continues to evolve alongside the practice of medicine, integrating modern diagnostic techniques while preserving the core principles of clinical reasoning and physical diagnosis that remain at the heart of patient care.

The comprehensive updates introduced in the thirteenth edition continue here, reflecting advancements in clinical education, patient care, and medical technology. With input from active clinicians and experienced educators, this edition responds to the changing needs of students and instructors across health care disciplines, ensuring it remains relevant, accessible, and clinically practical. A key focus of this edition is enhancing the usability and clinical application of the physical examination chapters, with refinements that help learners bridge the gap between textbook knowledge and real-world patient care.

This edition expands the **structured health history questions** to provide clearer guidance on how to ask effective, clinically relevant questions and interpret patient responses in ways that inform diagnosis and management. These revisions emphasize the connection between patient narratives, clinical reasoning, and differential diagnosis, helping learners move beyond memorization toward a deeper understanding of history-taking as a critical diagnostic tool. **Discussions of common symptoms** have also been refined, providing a stronger foundation for assessing patient complaints by integrating key historical features, physical exam findings, and considerations for differential diagnosis. Rather than presenting symptoms in isolation, this approach reinforces how clinicians analyze and synthesize information in practice. In addition, recognizing that many patients present with medical devices or prior surgical interventions that require **modifications to standard examination techniques**, this edition includes specific guidance on how to adapt the physical exam in these cases. These updates ensure that learners develop a flexible, patient-centered approach that accounts for real-world complexities in clinical assessment.

New content includes a dedicated chapter on **Point-of-Care Ultrasound (POCUS)** and updates to **Clinical Documentation and Presentation**, equipping learners with the tools to integrate technology-assisted diagnostics and effective communication into clinical workflows. The **Older Adults chapter**, now structured around the **Geriatric 5Ms framework**, provides a comprehensive approach to assessing aging patients, reinforcing the importance of age-specific considerations in physical diagnosis.

Beyond structural and content updates, terminology throughout the text has been refined to ensure clarity, accuracy, and consistency with modern medical language. Where historical medical terms and eponyms are still in use, they have been carefully retained for continuity while ensuring that descriptions are as precise and clinically useful as possible. The **Health Promotion and Counseling** sections have also been expanded, emphasizing patient-centered communication, disease prevention strategies, and clinical reasoning in primary care and specialty settings.

With these updates, the fourteenth edition of *Bates' Guide to Physical Examination and History Taking* remains a cornerstone resource in health care education, equipping students, educators, and practicing clinicians with the knowledge and skills necessary for effective, evidence-based, and

compassionate patient care. This edition upholds the highest standards in medical training, ensuring that future health care professionals are well prepared to integrate clinical knowledge, physical exam skills, and patient communication into their practice.

R.P. Soriano, MD
Lead Author and Editor

New Content and Features

The fourteenth edition introduces several new features and expanded content designed to enhance clinical learning and patient assessment skills. These updates improve both usability and content depth, ensuring that learners gain a strong foundation in physical examination, clinical reasoning, and diagnostic techniques.

Layout and Design Enhancements:

- The streamlined double-column layout for Unit 1 has been introduced to improve readability, allowing for a cleaner and more focused presentation of foundational content. Units 2 and 3 retain the familiar two-column layout, providing continuity and maintaining the traditional structure that many readers appreciate.
- We have revamped the overall layout to make navigation easier, ensuring that you can find the information you need quickly and efficiently. This improved design enhances your study experience by reducing the time spent searching for key topics.
- With new, high-quality images added throughout the book provide greater visual clarity, reinforcing important physical examination techniques and clinical findings. These images are designed to support and enhance the textual content, making complex concepts more intuitive.
- Additional tables have been incorporated to organize information more clearly. These tables help distill complex data into easily digestible formats, aiding in quicker comprehension and retention of critical facts.

Refinements in Language and Clinical Terminology:

- Language and terminology have been refined to ensure clarity, precision, and accessibility, reflecting best practices in clinical communication while maintaining consistency with modern health care education standards.

Enhancements to Regional Physical Examination Chapters:

- The inclusion of common symptoms with high-yield health history questions is aimed at enhancing clinical reasoning-focused interviews. These structured questions guide learners in asking the right questions during patient assessments, ultimately improving diagnostic accuracy.
- New sections on high-yield Point-of-Care Ultrasound (POCUS) techniques in selected regional physical examination chapters provide essential skills for modern diagnostic practices. Learning these techniques alongside time-honored physical exam skills will give learners a competitive edge in clinical settings and develop a more comprehensive diagnostic approach.
- We have added new tables on physical examination modifications for patients with clinical devices. These tables offer practical guidance on adapting exams for patients with special needs, ensuring comprehensive, patient-centered, and sensitive care.
- The new clinical reasoning-focused interpretation of sample documentations in regional chapters will help learners understand how to effectively document findings and think critically about assessments. This skill is crucial for developing accurate patient records and improving overall patient care.

- Updated recommendations in the Health Promotion and Counseling sections ensure you have access to the latest guidelines and best practices for patient care. Staying current with these recommendations is vital for providing the best possible advice and treatment to patients.

Specific Chapter Revisions and Additions:

- The addition of new chapters on POCUS and Documentation and Presentation expands the scope of the textbook, providing learners with essential modern techniques and skills that are increasingly important in clinical practice.
- The Genitourinary (GU) content has been reorganized, and the Musculoskeletal (MSK) chapter has been split, allowing for a more detailed and focused presentation of each area. This reorganization allows for a deeper understanding of each area, enhancing overall clinical knowledge and application.
- The introduction of the 5Ms framework in the Older Adults chapter enhances health history and assessment for older patients, offering a comprehensive approach to evaluating and managing geriatric care. This framework ensures a thorough and holistic approach to geriatric care, which is crucial given the aging population.

These updates and enhancements ensure that the fourteenth edition remains a comprehensive and indispensable resource for clinical skills training and education. With these improvements, learners will be well equipped to succeed in their clinical studies and future careers, confident in their ability to perform thorough physical examinations, apply clinical reasoning, and provide high-quality patient care.

ORGANIZATION

The book comprises three units: *Foundations of Health Assessment*, *Regional Examinations*, and *Special Populations*.

Unit 1 , Foundations of Health Assessment, focuses on essential skills and knowledge required for the clinical encounter. This unit with nine chapters begins with foundational skills crucial to the clinical setting, followed by effective interviewing, communication, and interpersonal skills. The subsequent chapters cover the health history, physical examination, and the processes of clinical reasoning, assessment, and planning. In addition, this unit includes new chapters on clinical documentation and presentation, as well as the basic principles and techniques of point-of-care ultrasound. Health maintenance and screening are also addressed, along with evaluating clinical evidence, ensuring a comprehensive foundation for effective patient assessment and clinical decision making.

Unit 2 , Regional Examinations, covers comprehensive regional examinations from head to toe. This unit with 18 chapters begins with an overview of the general survey, vital signs, and pain assessment, followed by cognition, behavior, and mental status. Subsequent chapters detail the examination of the skin, hair, and nails; head and neck; eyes; ears and nose; throat and oral cavity; thorax and lungs; cardiovascular system; and peripheral vascular system. The unit continues with the examination of the breasts and axillae, abdomen, anus and rectum, and the pelvis and genitourinary system, with separate chapters for different anatomical structures. The musculoskeletal system is covered in two chapters, focusing on the neck, shoulders, and upper extremities, and the lumbosacral spine, hip, and lower extremities, respectively. The unit concludes with a detailed examination of the

nervous system. Each chapter includes a review of relevant anatomy and physiology, common symptoms encountered in the health history, detailed descriptions and images of examination techniques, a sample written record, comparative tables of abnormalities, and extensive references from recent clinical literature. Important topics for health promotion and counseling are now placed at the end of each chapter to facilitate a more focused understanding of these complex issues.

Unit 3 , Special Populations, addresses health assessments across different stages of life, including three chapters on children (infancy through adolescence), pregnant individuals, and older adults.

ADDITIONAL RESOURCES

Bates' Pocket Guide to Physical Examination and History Taking

As a practical companion to *Bates' Guide to Physical Examination and History Taking, 14th Edition* , we highly recommend the *Bates' Pocket Guide to Physical Examination and History Taking, 10th Edition* . Together, these two resources are designed to support learners at every stage—whether laying the groundwork in the classroom or applying knowledge in real-time clinical settings.

The full textbook delivers comprehensive, in-depth instruction on examination techniques, clinical reasoning, and foundational principles of patient care. It is best suited for structured study, offering the detail and context needed to build lasting understanding and clinical confidence.

The Pocket Guide complements this by serving as a concise, accessible reference at the bedside or point of care. Its compact format makes it ideal for quick consultation during clinical encounters, while its clear summaries, updated algorithms, and new illness scripts help reinforce core concepts and support diagnostic reasoning in the moment. The continued inclusion of clinical algorithms reflects their enduring value and the consistently strong feedback from learners and clinicians alike.

Used together, the textbook and Pocket Guide form a dynamic and integrated learning system—helping learners bridge the gap between knowledge and practice with clarity, confidence, and ease.

Bates' Videos

The Bates' video series is an essential adjunct for mastering the many techniques of physical examination. Comprehensive videos bring clinical skills to life through clear, structured demonstrations organized into three integrated collections:

Physical Examination Series: This core series features 18 volumes of head-to-toe and systems-based physical examination videos. Updated and reorganized for clarity and consistency, each video emphasizes relevant anatomy, step-by-step examination techniques, and common pathologic findings. Learners observe experienced clinicians performing regional exams and applying techniques such as inspection, palpation, percussion, and auscultation—across both general and special populations.

Communication and Interpersonal Skills Series: This 27-video series highlights the essential human side of clinical practice. Learners watch clinicians demonstrate foundational communication and interpersonal skills, from building rapport and gathering information to navigating difficult

conversations. These skills are critical for effective, compassionate care and will serve as a lifelong foundation for professional growth.

OSCE Clinical Skills Series: Designed to strengthen clinical reasoning and exam readiness, this series includes 15 realistic, scenario-based patient encounters that mirror Objective Structured Clinical Examinations (OSCEs). Each video features a student evaluating a patient with a common presenting problem—such as chest pain, sore throat, or abdominal pain—interspersed with guiding questions and key teaching points. These videos help learners apply their knowledge in context and prepare for both formative assessments and real-world patient care.

Students are encouraged to use the textbook, pocket guide, and video series together as a cohesive learning system—studying the textbook to build foundational knowledge, using the pocket guide to support clinical application, and revisiting the videos to visually reinforce key techniques. Engaging with these resources repeatedly and at different stages of learning helps solidify understanding, sharpen skills, and build confidence in both the classroom and clinical environment.

The Bates' video series is available via Lippincott® Connect as the *Bates' Visual Guide to Physical Examination*. Purchase *Bates' Guide to Physical Examination and History Taking*, Lippincott® Connect version with Videos, for access to the videos, assessment, and other dynamic resources that enhance your learning.

Bates' Visual Guide to Physical Examination is available for purchase by institutions and enables seamless, concurrent access for all students, with options for institutions to subscribe to the full range of videos or select collections. Each video has a unique, stable URL that instructors can provide in their Learning Management Systems for easy student access.

Acknowledgments

Bates' Guide to Physical Examination and History Taking has always been a collaborative effort driven by a singular goal: to honor and support the lifelong learner—student, teacher, and practitioner—in mastering the evolving art and science of clinical medicine. This fourteenth edition continues this tradition, bringing together contributions from a diverse group of dedicated professionals committed to excellence in physical diagnosis and patient care. With this edition, *Dr. Rainier P. Soriano* assumes the role of lead author and editor, building on the solid foundation established by previous editions. He brings a deep commitment to integrating traditional physical examination with evolving best practices in clinical teaching.

Profound appreciation is extended to *Dr. Lynn S. Bickley*, who skillfully led the guide from the seventh through the thirteenth editions. Her vision and clarity helped shape generations of learners. Gratitude is also owed to *Dr. Peter L. Szilagyi*, whose contributions since the eighth edition have greatly enriched the content, and to *Dr. Richard M. Hoffman*, whose expertise since the twelfth edition has significantly advanced the integration of clinical evidence and health promotion strategies throughout the guide.

This edition also benefited greatly from the insights and contributions of a number of expert reviewers and advisors—active clinicians and educators who embody the intersection of bedside skills and clinical teaching. Their feedback ensures that the content remains accurate, relevant, and grounded in real-world practice. The contributions of the following esteemed colleagues are gratefully acknowledged: *George A. Alba, MD; Cara A. Brown, MD; Christopher T. Doughty, MD; Nicola A. Feldman, MD, MS; Rocco M. Ferrandino, MD, MSCR; Raisa Gao, MD; Sarah M. Gustafson, MD; Michael L. Herscher, MD; Alexander R. Lloyd, MD; Christopher C. Lo, MD; Stephen A. McCullough, MD; Bret P. Nelson, MD; Matthew E. Pollard, MD; and Bess M. Storch, MD.*

The production of the *Bates' Guide to Physical Examination and History Taking*, fourteenth edition is a complex endeavor requiring extraordinary coordination and meticulous attention to detail. Special thanks go to Freelance Development Editor *Kelly Horvath*, whose editorial precision and dedication brought coherence and excellence to every chapter. Additional heartfelt acknowledgments go to Development Editor *Deborah Bordeaux*, who has guided the development of the content; Senior Editorial Coordinator *Sean Hanrahan*, who has managed the editorial process; Marketing Manager *Kirsten Watrud*, and Production Project Manager *Catherine Ott* at Wolters Kluwer, whose oversight ensured the successful execution of every phase. Gratitude is also extended to Manager of Graphic Arts & Design *Stephen Druding*, Art Director of Illustration *Jennifer Clements* at Wolters Kluwer, and the team at *Body Scientific International*, for their creation of updated, high-quality illustrations; to photographer *Mike Stog* at DesignWorksVisualMedia and to *Liz Simmons Thibodeau* and her crew at Thibodeau Media Group, who have delivered exceptional photography and high-quality media production; and to *Aptara* for their expert handling of the digital production process. Finally, and most importantly, heartfelt thanks to Acquisitions Editor *Matt Hauber*, whose leadership and vision have ensured that the *Bates* suite of teaching materials continues to meet the evolving needs of today's learners and educators.

The talent, insight, and dedication of all those involved in this edition uphold the long-standing tradition of excellence that has made the *Bates' Guide to Physical Examination and History Taking*, reaffirming its role as the premier resource in the teaching and learning of clinical skills.

Contents

[*Faculty Contributors*](#)

[*Preface*](#)

[*New Content and Features*](#)

[*Acknowledgments*](#)

[*List of Tables*](#)

UNIT 1

Foundations of Health Assessment

CHAPTER 1

Foundational Skills Essential to the Clinical Encounter

THE CLINICAL ENCOUNTER

APPROACH TO THE CLINICAL ENCOUNTER

STRUCTURE AND SEQUENCE OF THE CLINICAL ENCOUNTER

[Stage 1: Initiating the Encounter](#)

[Stage 2: Gathering Information](#)

[Stage 3: Performing the Physical Examination](#)

[Stage 4: Explaining and Planning](#)

[Stage 5: Closing the Encounter](#)

DISPARITIES IN HEALTH CARE

[Social Determinants of Health](#)

[Racism and Bias](#)

[Cultural Humility](#)

OTHER MAJOR CONSIDERATIONS

[Spirituality](#)

[Medical Ethics](#)

[Decisional Capacity](#)

[Approach to a Clinical Ethical Dilemma](#)

REFERENCES

CHAPTER 2

Interviewing, Communication, and Interpersonal Skills

ORGANIZING PATIENT–CLINICIAN COMMUNICATION STRATEGIES

ESTABLISHING THE FOUNDATION OF COMMUNICATION

Active (Attentive) Listening

BUILDING THE INTERACTION

Guided Questioning

STRENGTHENING THE RELATIONSHIP

Empathic Responses

Summarization

Transitions

Validation

Partnering

Empowering the Patient

Reassurance

ADDRESSING SENSITIVE TOPICS

PRACTICING LEGAL AND ETHICAL COMMUNICATION

Informed Consent

Collaborating with a Medical Interpreter

USING ADVANCED COMMUNICATION STRATEGIES

Appropriate Verbal Communication

Appropriate Nonverbal Communication

NAVIGATING COMPLEX DISCUSSIONS

Disclosing Serious News

Advance Care Planning

ENCOURAGING PATIENT SELF-EFFICACY

Motivational Interviewing (MI)

ENHANCING INTERPROFESSIONAL COMMUNICATION

MANAGING CHALLENGING PATIENT SITUATIONS AND BEHAVIORS

BEING PATIENT-CENTERED IN COMPUTERIZED CLINICAL SETTINGS

LEARNING COMMUNICATION SKILLS FROM STANDARDIZED PATIENTS

REFERENCES

CHAPTER 3

Health History

HEALTH HISTORY

Different Kinds of Health Histories

Determining the Scope of Your Patient Assessment: Comprehensive or Focused?

Subjective versus Objective Data

COMPONENTS OF THE ADULT HEALTH HISTORY

Initial Patient Information

Gather Information about the Patient's Chief Concern or Presenting Concern

Gather Information about the Patient's History of Present Illness

Gather Information about the Patient's Past Medical History

Gather Information about the Patient's Family History

Gather Information about the Patient's Personal and Social History

Review of Systems

RECORDING YOUR FINDINGS

MODIFICATION OF THE CLINICAL INTERVIEW FOR VARIOUS CLINICAL SETTINGS

REFERENCES

CHAPTER 4

Physical Examination

ROLE OF THE PHYSICAL EXAMINATION IN THE ERA OF TECHNOLOGY

CONDUCTING CARDINAL TECHNIQUES OF EXAMINATION

Auscultating Through Clothing in Clinical Practice

BEGINNING THE PHYSICAL EXAMINATION

Reflecting on Your Approach

Optimizing the Examination Environment

Preparing Your Equipment

Enhancing Patient Comfort

Observing Standard and Universal Precautions

Optimizing the Sequence, Scope, and Positioning of Examination

Sequence of Examination

Positioning of Examination

Scope of Examination

PERFORMING A HEAD-TO-TOE PHYSICAL EXAMINATION

ADAPTING THE PHYSICAL EXAMINATION: SPECIFIC PATIENT CONDITIONS

RECORDING YOUR FINDINGS

REFERENCES

CHAPTER 5

Clinical Reasoning

DEVELOPING A DIFFERENTIAL DIAGNOSIS: MASTERING CLINICAL REASONING

CLINICAL REASONING PROCESS

BASIC STRUCTURE OF THE CLINICAL REASONING PROCESS

[Step 1: Gathering Initial Patient Information](#)

[Step 2: Organizing and Interpreting Clinical Information](#)

[Step 3: Synthesizing Clinical Information and Developing the Problem Representation](#)

[Step 4: Generating Hypotheses by Searching for the Probable Cause of the Findings](#)

[Step 5: Testing Hypotheses and Establishing a Working Diagnosis](#)

[Step 6: Planning the Diagnostic and Treatment Strategy](#)

CLINICAL DIAGNOSTIC ERRORS

CLINICAL REASONING: DOCUMENTATION

REFERENCES

CHAPTER 6

Clinical Documentation and Oral Presentation

NAVIGATING COMMUNICATION IN HEALTH CARE

[Written Documentation in Electronic Health Records](#)

[Oral Presentations in Health Care](#)

DOCUMENTING HISTORY AND PHYSICAL EXAMINATION NOTES

[Initial Information](#)

[Source and Reliability](#)

[Chief Concern](#)

[History of Present Illness](#)

[Prior Health History](#)

[Family History](#)

[Social History](#)

[Review of Systems](#)

[Physical Examination](#)

[Assessment and Plan](#)

CONDUCTING ORAL PRESENTATIONS FOR NEW PATIENTS

DOCUMENTING PROGRESS NOTES

ADDRESSING SPECIAL CONSIDERATIONS IN CLINICAL DOCUMENTATION

REFERENCES

CHAPTER 7

Health Maintenance and Screening

CONCEPT OF PREVENTIVE CARE

GUIDELINE RECOMMENDATIONS

[United States Preventive Services Task Force Approach](#)

SCREENING

BEHAVIORAL COUNSELING

[Stages of Behavioral Change Model](#)

Structured Counseling Models: Five As and FRAMES

IMMUNIZATION

SCREENING GUIDELINES FOR ADULTS

Screening for Substance Use Disorders, Including Misuse of Prescription and Illicit Drugs

Screening for Intimate Partner Violence in Females of Reproductive Age

Screening for Unhealthy Alcohol Use

Screening for Tobacco Use

Screening for Sexually Transmitted Infections: Syphilis, Chlamydia, and Gonorrhea

Screening for Human Immunodeficiency Virus

Screening for Prediabetes and Type 2 Diabetes

COUNSELING GUIDELINES FOR ADULTS

Counseling for Unhealthy Alcohol Use

Counseling for Tobacco Smoking Cessation

Counseling on Sexually Transmitted Infections

IMMUNIZATION GUIDELINES FOR ADULTS

Influenza Vaccine

Pneumococcal Vaccine

Respiratory Syncytial Virus Vaccine

COVID-19 Vaccine

Mpox Vaccine

Varicella Vaccine

Herpes Zoster Vaccine

Tetanus, Diphtheria, Pertussis Vaccine

Human Papillomavirus Vaccine

Hepatitis A Vaccine

Hepatitis B Vaccine

PREVENTIVE MEDICATION GUIDELINES

Prevention of Human Immunodeficiency Virus Infection: Pre-exposure Prophylaxis

PREVENTIVE CARE IN SPECIAL POPULATIONS

DISEASE-SPECIFIC RECOMMENDATIONS

REFERENCES

CHAPTER 8

Evaluating Clinical Evidence

INTRODUCTION

USING ELEMENTS OF THE HISTORY AND PHYSICAL EXAMINATION AS DIAGNOSTIC TESTS

EVALUATING DIAGNOSTIC TESTS: VALIDITY

Sensitivity and Specificity

Predictive Values

Prevalence of Disease

Likelihood Ratios

APPLYING CONCEPTS TO EVALUATING ABDOMINAL PAIN

APPLYING CONCEPTS TO SCREENING TESTS

[Fagan Nomogram](#)

[Natural Frequencies](#)

EVALUATING DIAGNOSTIC TESTS: REPRODUCIBILITY

[Kappa Score](#)

[Precision](#)

CRITICALLY APPRAISING THE CLINICAL EVIDENCE

[Are the Results Valid?](#)

[What Are the Results?](#)

[How Can You Apply the Results to Patient Care?](#)

COMMUNICATING CLINICAL EVIDENCE TO PATIENTS

REFERENCES

CHAPTER 9

Basic Principles and Techniques: Point-of-Care Ultrasound

POINT-OF-CARE ULTRASOUND

[Goal of the Chapter](#)

[Enhancing Clinical Assessment: Insonation](#)

SOUND WAVES AND ULTRASOUND BASICS

[Sound Waves](#)

[Acoustic Impedance in Ultrasound Imaging](#)

ULTRASOUND EQUIPMENT AND TECHNIQUE

[Ultrasound Transducers](#)

[Image Acquisition](#)

UNDERSTANDING IMAGING MODES

[B-Mode \(Brightness Mode\)](#)

[M-Mode \(Motion Mode\)](#)

[Doppler Modes, Color, and Pulsed Wave](#)

ULTRASOUND IMAGE OPTIMIZATION ("KNOBOLOGY")

[Machine Settings and Adjustments](#)

REFERENCES

UNIT 2

Regional Examinations

CHAPTER 10

General Survey, Vital Signs, and Pain

HEALTH HISTORY: GENERAL APPROACH

Fatigue and Weakness

Fever and Night Sweats

Weight Change

Pain

PHYSICAL EXAMINATION: GENERAL APPROACH

TECHNIQUES OF EXAMINATION

Perform a General Survey

Measure Height and Weight

Assess Vital Signs

Measure Blood Pressure

Examine Arterial Pulses for Rate and Rhythm

Observe Respiratory Rate, Rhythm, Depth, and Effort

Measure Core Body Temperature

Assess for Acute and Chronic Pain

Consider Health Disparities in Pain

SPECIAL TECHNIQUES AND MANEUVERS

Measure Orthostatic Blood Pressures

Special Patient-Related Situations

Out-of-Office and Self-Monitoring of Blood Pressure

RECORDING YOUR FINDINGS

HEALTH PROMOTION AND COUNSELING: EVIDENCE AND RECOMMENDATIONS

Achieving Optimal Weight

REFERENCES

CHAPTER 11

Cognition, Behavior, and Mental Status

ANATOMY AND PHYSIOLOGY

HEALTH HISTORY: GENERAL APPROACH

Anxiety and Excessive Worrying

Depressed Mood

Memory Problems

Medically Unexplained Symptoms

PHYSICAL EXAMINATION: GENERAL APPROACH

TECHNIQUES OF EXAMINATION

Assess Appearance and Behavior Including Level of Consciousness

Assess Speech and Language

Assess Mood

Assess Thought Process and Perceptions

Assess Cognitive Function

[Assess Higher Cognitive Functions](#)

RECORDING YOUR FINDINGS

HEALTH PROMOTION AND COUNSELING: EVIDENCE AND RECOMMENDATIONS

[Screening for Depression](#)

[Screening for Anxiety Disorders](#)

[Screening for Neurocognitive Disorders](#)

REFERENCES

CHAPTER 12

Skin, Hair, and Nails

ANATOMY AND PHYSIOLOGY

[Skin](#)

[Hair](#)

[Nails](#)

[Pilosebaceous Glands and Sweat Glands](#)

HEALTH HISTORY: GENERAL APPROACH

[Skin Lesions](#)

[Rashes and Itching \(Pruritus\)](#)

[Hair Loss or Alopecia](#)

[Nail Changes](#)

DESCRIBING SKIN LESIONS

[Primary Lesions](#)

[Other Features of Skin Lesions](#)

PHYSICAL EXAMINATION: GENERAL APPROACH

[Skin Examination Setup](#)

TECHNIQUES OF EXAMINATION

[Examine the Hair and Scalp—Seated](#)

[Examine the Head and Neck—Seated](#)

[Examine the Upper Back—Seated](#)

[Examine the Shoulders, Arms, and Hands—Seated](#)

[Examine the Chest and Abdomen—Seated](#)

[Examine the Anterior Thighs and Legs—Seated](#)

[Examine the Feet and Toes—Seated](#)

[Examine the Lower Back, Posterior Thighs, and Legs—Standing](#)

[Integrate a Full-Body Skin Examination](#)

SPECIAL TECHNIQUES AND MANEUVERS

[Instructing the Patient in Skin Self-Examination](#)

[Examining the Patient with Hair Loss](#)

[Evaluating the Patient Who Is Confined to a Bed](#)

[Modifications in Physical Examinations: Best Practices for Specialized Patient Populations](#)

RECORDING YOUR FINDINGS

POINT-OF-CARE ULTRASOUND EXAMINATION

[Distinguishing Between Cellulitis and Abscess](#)

HEALTH PROMOTION AND COUNSELING: EVIDENCE AND RECOMMENDATIONS

[Skin Cancer Epidemiology](#)

[Skin Cancer Prevention](#)

[Clinician Skin Cancer Screening](#)

[Skin Self-Examination](#)

REFERENCES

CHAPTER 13

Head and Neck

ANATOMY AND PHYSIOLOGY

[Head and Neck](#)

[Great Vessels](#)

[Midline Structures and Thyroid Gland](#)

[Cervical Lymph Nodes](#)

HEALTH HISTORY: GENERAL APPROACH

[Neck or Thyroid Mass or Lump](#)

PHYSICAL EXAMINATION: GENERAL APPROACH

TECHNIQUES OF EXAMINATION

[Examine the Hair](#)

[Examine the Scalp](#)

[Examine the Skull](#)

[Inspect Facial Contours](#)

[Inspect the Skin on the Head and Face](#)

[Palpate the Cervical Lymph Nodes](#)

[Examine the Trachea](#)

[Inspect the Thyroid Gland](#)

[Palpate the Thyroid Gland: Posterior Approach](#)

[Palpate the Thyroid Gland: Anterior Approach](#)

[Examine the Carotid Arteries and Jugular Veins](#)

RECORDING YOUR FINDINGS

HEALTH PROMOTION AND COUNSELING: EVIDENCE AND RECOMMENDATIONS

[Screening for Thyroid Dysfunction](#)

[Screening for Thyroid Cancer](#)

REFERENCES

CHAPTER 14

Eyes

ANATOMY AND PHYSIOLOGY

[Orbital Anatomy](#)

[Anterior Ocular Structures](#)
[Lacrimal Apparatus](#)
[Optical Structures](#)
[Posterior Ocular Structures](#)
[Visual Pathway and Fields](#)
[Autonomic Nerve Supply to the Eyes](#)
[Extraocular Muscles](#)

HEALTH HISTORY: GENERAL APPROACH

[Blurred Vision](#)
[Eye Redness or Pain](#)

PHYSICAL EXAMINATION: GENERAL APPROACH

TECHNIQUES OF EXAMINATION

[Test Visual Acuity](#)
[Assess Visual Fields](#)
[Test Color Vision and Contrast Sensitivity](#)
[Assess Eye Position and Alignment](#)
[Inspect the Eyebrows](#)
[Inspect the Eyelids and Eyelashes](#)
[Inspect the Lacrimal Apparatus](#)
[Inspect the Conjunctivae and Sclerae](#)
[Inspect the Cornea, Iris, and Lens](#)
[Inspect the Pupils](#)
[Assess the Pupillary Light Reactions](#)
[Assess the Pupillary Near Reaction](#)
[Assess Conjugate Movements](#)
[Test Extraocular Muscle Movements](#)
[Test for Convergence](#)
[Perform a Funduscopy Examination \(Direct Ophthalmoscopy\)](#)

SPECIAL TECHNIQUES AND MANEUVERS

[Assessing Eye Protrusion \(Proptosis or Exophthalmos\)](#)
[Examining for Nasolacrimal Duct Obstruction](#)
[Managing Foreign Body in the Eye](#)
[Testing for Functional Impairment of the Optic Nerves](#)
[Modifications in Physical Examinations: Best Practices for Specialized Patient Populations](#)

RECORDING YOUR FINDINGS

HEALTH PROMOTION AND COUNSELING: EVIDENCE AND RECOMMENDATIONS

[Visual Impairment](#)
[Screening for Glaucoma](#)
[UV-Related Eye Injuries](#)

REFERENCES

CHAPTER 15

Ears and Nose

EAR: ANATOMY AND PHYSIOLOGY

External Ear

Middle Ear

Inner Ear

Hearing Pathways

Vestibular System and Equilibrium

NOSE: ANATOMY AND PHYSIOLOGY

Nasal Septum

Turbinates and Meatuses

Paranasal Sinuses

Olfaction Pathway

HEALTH HISTORY: GENERAL APPROACH

Hearing Loss

Earache and Ear Discharge

Ringing in the Ear (Tinnitus)

Dizziness and Lightheadedness

Nasal Discharge (Rhinorrhea) and Nasal Congestion

Nosebleed (Epistaxis)

PHYSICAL EXAMINATION: GENERAL APPROACH

EAR: TECHNIQUES OF EXAMINATION

Examine the Auricle and Surrounding Tissue and Palpate the Tragus and Mastoid

Examine the Ear Canals and Tympanic Membranes (Otoscopy)

Test Auditory Acuity (Gross Hearing)

Test for Conductive versus Sensorineural Hearing Loss

EAR: SPECIAL TECHNIQUES AND MANEUVERS

Modifications in Physical Examinations: Best Practices for Specialized Patient Populations

NOSE AND PARANASAL SINUS: TECHNIQUES OF EXAMINATION

Inspect the Anterior and Inferior Surfaces of the Nose

Test for Nasal Obstruction

Inspect the Nasal Cavity and Mucosa

Inspect the Nasal Septum

Inspect the Nasal Turbinates and Meatuses

Palpate the Paranasal Sinuses

RECORDING YOUR FINDINGS

HEALTH PROMOTION AND COUNSELING: EVIDENCE AND RECOMMENDATIONS

Screening for Hearing Loss

REFERENCES

CHAPTER 16

Throat and Oral Cavity

ANATOMY AND PHYSIOLOGY

Mouth, Gingiva, and Teeth

[Tongue](#)

[Gustation Pathway](#)

[Pharynx](#)

HEALTH HISTORY: GENERAL APPROACH

[Sore Throat](#)

[Bleeding or Swollen Gums](#)

[Hoarseness](#)

[Malodorous Breath \(Halitosis\)](#)

PHYSICAL EXAMINATION: GENERAL APPROACH

TECHNIQUES OF EXAMINATION

[Inspect the Lips and Oral Mucosa](#)

[Palpate the Oral Mucosa](#)

[Inspect the Gingiva and Teeth](#)

[Inspect the Roof and Floor of the Mouth](#)

[Inspect the Tongue](#)

[Test the Hypoglossal Nerve](#)

[Palpate the Tongue](#)

[Inspect the Soft Palate, Anterior and Posterior Pillars, Uvula, Tonsils, and Pharynx](#)

[Test the Vagus Nerve](#)

SPECIAL TECHNIQUES AND MANEUVERS

[Modifications in Physical Examinations: Best Practices for Specialized Patient Populations](#)

RECORDING YOUR FINDINGS

HEALTH PROMOTION AND COUNSELING: EVIDENCE AND RECOMMENDATIONS

[Oral Health](#)

[Screening for Oral and Pharyngeal Cancer](#)

REFERENCES

CHAPTER 17

Thorax and Lungs

ANATOMY AND PHYSIOLOGY

[Surface Anatomy: Vertical Axis](#)

[Surface Anatomy: Chest Circumference](#)

[Lungs, Fissures, and Lobes](#)

[Trachea and Major Bronchi \(Tracheobronchial Tree\)](#)

[Pleurae](#)

[Mechanics and Physiology of Breathing](#)

[Breath Sounds \(Lung Sounds\)](#)

HEALTH HISTORY: GENERAL APPROACH

[Shortness of Breath \(Dyspnea\) and Wheezing](#)

[Cough](#)

[Blood-Streaked Sputum \(Hemoptysis\)](#)

[Chest Pain/Discomfort](#)

[Daytime Sleepiness, Snoring, and Disordered Sleep](#)

PHYSICAL EXAMINATION: GENERAL APPROACH

TECHNIQUES OF EXAMINATION

[Survey the Thorax and Respiration](#)

[Inspect the Thorax](#)

[Palpate the Thorax](#)

[Assess for Thoracic Expansion \(Lung Excursion\)](#)

[Assess for Tactile Fremitus](#)

[Percuss the Thorax](#)

[Posterior Chest](#)

[Anterior Chest](#)

[Identify Extent of Diaphragmatic Excursion](#)

[Auscultate the Lungs for Breath Sounds](#)

[Auscultate for Adventitious \(Added\) Sounds](#)

[Assess Transmitted Voice Sounds](#)

SPECIAL TECHNIQUES AND MANEUVERS

[Clinically Assess Pulmonary Function](#)

[Assess Forced Expiratory Time](#)

[Identify a Fractured Rib](#)

[Modifications in Physical Examinations: Best Practices for Specialized Patient Populations](#)

RECORDING YOUR FINDINGS

POINT-OF-CARE ULTRASOUND EXAMINATION

[Detecting Congestive Heart Failure, Pneumothorax, and Pleural Effusion](#)

HEALTH PROMOTION AND COUNSELING: EVIDENCE AND RECOMMENDATIONS

[Lung Cancer](#)

[Latent Tuberculosis](#)

[Obstructive Sleep Apnea](#)

REFERENCES

CHAPTER 18

Cardiovascular System

ANATOMY AND PHYSIOLOGY

[Mediastinum, Heart, and Great Vessels](#)

[Conduction System](#)

[Heart as a Pump](#)

[Cardiac Circulation](#)

[Cardiac Cycle](#)

[Arterial Pulses and Blood Pressure](#)

[Jugular Venous Pressure and Pulsations](#)

[Relation of Cardiac Findings to the Chest Wall](#)

[Heart Sounds](#)

[Heart Murmurs](#)

HEALTH HISTORY: GENERAL APPROACH

[Chest Pain](#)

[Palpitations](#)

[Shortness of Breath](#)

[Swelling \(Edema\)](#)

[Fainting \(Syncope\)](#)

PHYSICAL EXAMINATION: GENERAL APPROACH

TECHNIQUES OF EXAMINATION

[Measure Blood Pressure and Heart Rate](#)

[Identify Jugular Venous Pressure](#)

[Auscultate for Carotid Bruits](#)

[Palpate the Carotid Arteries](#)

[Inspect the Precordium](#)

[Palpate the Precordium](#)

[Auscultate the Precordium](#)

[Auscultate for S1 and S2](#)

[Auscultate for Splitting of Heart Sounds](#)

[Auscultate for Heart Murmurs](#)

SPECIAL TECHNIQUES AND MANEUVERS

[Bedside Maneuvers to Identify Murmurs and Heart Failure](#)

[Modifications in Physical Examinations: Best Practices for Specialized Patient Populations](#)

RECORDING YOUR FINDINGS

POINT-OF-CARE ULTRASOUND EXAMINATION

[Assessing Left Ventricular Systolic Function](#)

HEALTH PROMOTION AND COUNSELING: EVIDENCE AND RECOMMENDATIONS

[Background](#)

[Screening for Hypertension](#)

[Initiating Statin Therapy for Primary Prevention](#)

[Promoting Lifestyle Changes and Risk Factor Modification](#)

[Addressing Excessive Dietary Sodium](#)

REFERENCES

CHAPTER 19

Peripheral Vascular System

ANATOMY AND PHYSIOLOGY

[Arterial System](#)

[Venous System](#)

[Lymphatic System](#)

[Transcapillary Fluid Exchange](#)

HEALTH HISTORY: GENERAL APPROACH

[Pain and/or Swelling of Legs with Intermittent Claudication](#)

[Pain and/or Swelling in Legs without Claudication](#)

[Abdominal, Flank, or Back Pain](#)

PHYSICAL EXAMINATION: GENERAL APPROACH

TECHNIQUES OF EXAMINATION

[Inspect the Upper Extremities](#)

[Palpate the Radial Pulse](#)

[Palpate the Brachial Pulse](#)

[Palpate for Epitrochlear Nodes](#)

[Examine the Abdominal Aorta](#)

[Palpate the Superficial Inguinal Nodes](#)

[Inspect the Lower Extremities](#)

[Assess the Temperature of the Feet and Legs](#)

[Palpate the Femoral Pulse](#)

[Palpate the Popliteal Pulse](#)

[Palpate the Posterior Tibial Pulse](#)

[Palpate the Dorsalis Pedis Pulse](#)

[Palpate for Pitting Edema](#)

[Palpate for Venous Tenderness or Cords](#)

SPECIAL TECHNIQUES AND MANEUVERS

[Assess for Peripheral Arterial Disease Using the Ankle–Brachial Index](#)

[Evaluate Arterial Perfusion of the Hand Using the Allen Test](#)

[Modifications in Physical Examinations: Best Practices for Specialized Patient Populations](#)

RECORDING YOUR FINDINGS

POINT-OF-CARE ULTRASOUND EXAMINATION

[Detecting Nonpalpable Pulses](#)

[Detecting Deep Venous Thrombosis](#)

HEALTH PROMOTION AND COUNSELING: EVIDENCE AND RECOMMENDATIONS

[Screening for Lower Extremity Peripheral Artery Disease](#)

[Screening for Abdominal Aortic Aneurysm](#)

REFERENCES

CHAPTER 20

Breasts and Axillae

ANATOMY AND PHYSIOLOGY

[Breast](#)

[Axilla](#)

HEALTH HISTORY: GENERAL APPROACH

[Breast Lump or Mass](#)

[Breast Discomfort or Pain](#)

[Nipple Discharge](#)

PHYSICAL EXAMINATION: GENERAL APPROACH

TECHNIQUES OF EXAMINATION

[Inspect the Breasts of Individuals Assigned Female at Birth in Four Views](#)

[Palpate the Breasts](#)

[Inspect the Axillae](#)

[Palpate the Axillary Lymph Nodes](#)

[Inspect the Breasts of Individuals Assigned Male at Birth](#)

SPECIAL TECHNIQUES AND MANEUVERS

[Modifications in Physical Examinations: Best Practices for Specialized Patient Populations](#)

RECORDING YOUR FINDINGS

HEALTH PROMOTION AND COUNSELING: EVIDENCE AND RECOMMENDATIONS

[Breast Cancer in Individuals Assigned Female at Birth](#)

[Breast Cancer in Individuals Assigned Male at Birth](#)

REFERENCES

CHAPTER 21

Abdomen

ANATOMY AND PHYSIOLOGY

[Abdominal Division Methods and Contents](#)

[Physiology of the Abdominal Organs](#)

HEALTH HISTORY: GENERAL APPROACH

[Abdominal Pain](#)

[Epigastric Pain](#)

[Right Upper Quadrant Abdominal Pain](#)

[Left Upper Quadrant Abdominal Pain](#)

[Right Lower Quadrant Abdominal Pain](#)

[Left Lower Quadrant Abdominal Pain](#)

[Hypogastric \(Suprapubic\) Pain](#)

[Nausea and Vomiting](#)

[Hematemesis](#)

[Hematochezia and Melena](#)

[Dysphagia and/or Odynophagia](#)

[Diarrhea](#)

[Constipation](#)

[Jaundice](#)

PHYSICAL EXAMINATION: GENERAL APPROACH

TECHNIQUES OF EXAMINATION

[Inspect the Abdomen](#)

[Auscultate for Bowel Sounds](#)

[Auscultate for Abdominal Bruits](#)

[Percuss the Abdomen](#)

[Palpate Lightly in All Four Abdominal Quadrants](#)

[Palpate Deeply in All Four Abdominal Quadrants](#)

[Estimate Liver Size by Percussion](#)

[Palpate the Liver Edge](#)

[Percuss for Splenic Enlargement](#)

[Palpate for the Splenic Edge](#)

[Palpate for Aortic Pulsations](#)

SPECIAL TECHNIQUES AND MANEUVERS

[Assessing Possible Peritonitis](#)

[Assessing Possible Ascites](#)

[Identifying Abdominal Organs and Masses in the Presence of Ascites](#)

[Assessing for Abdominal Wall Mass](#)

[Assessing Possible Appendicitis](#)

[Assessing Possible Acute Cholecystitis](#)

[Assessing Possible Renal Colic or Pyelonephritis](#)

[Assessing Ventral Hernias](#)

[Assessing for Urinary Bladder Distention](#)

[Modifications in Physical Examinations: Best Practices for Specialized Patient Populations](#)

RECORDING YOUR FINDINGS

POINT-OF-CARE ULTRASOUND EXAMINATION

[Ultrasound Technique](#)

[Assessing the Liver Size](#)

[Evaluating the Gallbladder](#)

[Evaluating for Intraperitoneal Free Fluid \(FF\)](#)

HEALTH PROMOTION AND COUNSELING: EVIDENCE AND RECOMMENDATIONS

[Viral Hepatitis A](#)

[Viral Hepatitis B](#)

[Viral Hepatitis C](#)

[Colorectal Cancer](#)

REFERENCES

CHAPTER 22

Anus and Rectum

ANATOMY AND PHYSIOLOGY

HEALTH HISTORY: GENERAL APPROACH

[Bloody Stool \(Hematochezia\)](#)

[Pain on Defecation](#)

[Anal and Perianal Lesions](#)

PHYSICAL EXAMINATION: GENERAL APPROACH

TECHNIQUES OF EXAMINATION

[Properly Position the Patient](#)

[Inspect the Sacrococcygeal and Perianal Areas](#)

[Inspect the Anus](#)

[Perform a Digital Rectal Examination](#)

SPECIAL TECHNIQUES AND MANEUVERS

[Perform a Prostatic Examination \(When Indicated\)](#)

[Perform a Rectovaginal Examination \(When Indicated\)](#)

RECORDING YOUR FINDINGS

REFERENCES

CHAPTER 23

Pelvis and Genitourinary System: Penis, Scrotum, and Prostate

ANATOMY AND PHYSIOLOGY

Pelvis

Urogenital System

Prostate

Pelvic Floor Muscles

Groin or Inguinal Area

Blood, Lymphatic, and Nerve Supplies

Micturition Cycle

Sexual Development and Function

HEALTH HISTORY: GENERAL APPROACH

Lower Urinary Tract Storage Symptoms

Lower Urinary Tract Voiding Symptoms

Lower Urinary Tract Postmicturition Symptoms

Urinary Incontinence

Penile Discharge or Lesions

Scrotal or Testicular Pain, Swelling, or Lesions

Erectile Dysfunction

PHYSICAL EXAMINATION: GENERAL APPROACH

TECHNIQUES OF EXAMINATION

Inspect the Penile Skin, Prepuce, and Glans

Inspect the Urethral Meatus

Palpate the Shaft of the Penis

Inspect the Scrotum

Assess for Groin Hernias

Palpate Each Testis

Palpate the Epididymis

Examine the Spermatic Cord

SPECIAL TECHNIQUES AND MANEUVERS

Evaluating a Suspected Inguinal Hernia

Evaluating a Suspected Femoral Hernia

Assessing for Scrotal Contents by Transillumination

Performing a Testicular Self-Examination

Modifications in Physical Examinations: Best Practices for Specialized Patient Populations

Palpate the Posterior Surface of the Prostate Gland

Assess the Prostate's Size, Shape, Symmetry, Mobility, and Consistency

RECORDING YOUR FINDINGS

HEALTH PROMOTION AND COUNSELING: EVIDENCE AND RECOMMENDATIONS

Prostate Cancer

REFERENCES

CHAPTER 24

Pelvis and Genitourinary System: Vulva, Vagina, Uterus, and Adnexa

ANATOMY AND PHYSIOLOGY

Pelvis

Pelvic Floor

Vulva

Vagina

Uterus

Cervix

Adnexa

Micturition Cycle

Menstrual Cycle

Menarche and Menses

Menopause

HEALTH HISTORY: GENERAL APPROACH

Painful Menstruation (Dysmenorrhea)

Absence of Menses (Amenorrhea)

Abnormal Uterine Bleeding

Pelvic Pain

Vulvovaginal Symptoms

Urinary Incontinence

PHYSICAL EXAMINATION: GENERAL APPROACH

Positioning

Prepare the Equipment

TECHNIQUES OF EXAMINATION

Assess Sexual Maturity (Adolescents)

Inspect the Vulva

Insert the Vaginal Speculum

Inspect the Cervix

Inspect the Vagina

Obtain Specimens for Cervical Cytology (Pap Smears)

Perform a Bimanual Examination

Assess the Pelvic Floor Muscles for Strength and Tenderness

Perform a Rectovaginal Examination (if Indicated)

SPECIAL TECHNIQUES AND MANEUVERS

Assessing Labial Swelling

Assessing for Urethritis

Assessing for Suspected Inguinal Hernia

Assessing for Suspected Femoral Hernia

Modifications in Physical Examinations: Best Practices for Specialized Patient Populations

RECORDING YOUR FINDINGS

POINT-OF-CARE ULTRASOUND EXAMINATION

Estimating Bladder Volume

HEALTH PROMOTION AND COUNSELING: EVIDENCE AND RECOMMENDATIONS

Cervical Cancer

Menopause and Hormone Replacement Therapy

Urinary Incontinence

REFERENCES

CHAPTER 25

Musculoskeletal System: Neck, Shoulders, and Upper Extremities

ANATOMY AND PHYSIOLOGY: GENERAL MUSCULOSKELETAL SYSTEM

Vertebral Spine

Sacroccocygeal Spine

Joints

HEALTH HISTORY: GENERAL APPROACH

Musculoskeletal Problems

PHYSICAL EXAMINATION: GENERAL APPROACH

Inspection (Look)

Palpation (Feel)

Range of Motion (Move)

Special Maneuvers (Stress)

REGIONAL MUSCULOSKELETAL EXAMINATIONS

ANATOMY: TEMPOROMANDIBULAR JOINT

HEALTH HISTORY: GENERAL APPROACH

Jaw Pain

TECHNIQUES OF EXAMINATION

Inspect the Face and Temporomandibular Joint

Palpate the Temporomandibular Joint

Palpate the Muscles of Mastication

Assess Temporomandibular Joint Range of Motion

ANATOMY: CERVICAL SPINE (NECK)

HEALTH HISTORY: GENERAL APPROACH

Neck Pain or Stiffness

TECHNIQUES OF EXAMINATION

Inspect the Neck and Posture

Palpate the Muscles of the Neck

Palpate the Neck and Surrounding Structures

Assess Cervical Spine (Neck) Range of Motion

Perform Special Maneuvers for the Cervical Spine (If Indicated)

ANATOMY: SHOULDER

HEALTH HISTORY: GENERAL APPROACH

Shoulder Pain

TECHNIQUES OF EXAMINATION

Inspect the Shoulder and Associated Structures

Palpate the Shoulder and Associated Structures

Assess Shoulder Range of Motion

Perform Special Maneuvers for the Shoulder (If Indicated)

ANATOMY: ELBOW

HEALTH HISTORY: GENERAL APPROACH

Elbow Pain

TECHNIQUES OF EXAMINATION

Inspect the Elbow Joint

Palpate the Elbow Joint and Adjoining Structures

Assess Elbow Range of Motion

Perform Special Maneuvers for the Elbow Joint (If Indicated)

ANATOMY: WRIST AND HAND

Wrist Structure

Hand Structure

HEALTH HISTORY: GENERAL APPROACH

Wrist and Hand Pain

TECHNIQUES OF EXAMINATION

Inspect the Wrist

Inspect the Hand

Palpate the Wrist

Palpate the Hand

Assess Wrist, Fingers, and Thumb Range of Motion

Perform Special Maneuvers for the Wrist and Hand (If Indicated)

RECORDING YOUR FINDINGS

HEALTH PROMOTION AND COUNSELING: EVIDENCE AND RECOMMENDATIONS

REFERENCES

CHAPTER 26

Musculoskeletal System: Lumbosacral Spine, Hips, and Lower Extremities

ANATOMY: LUMBOSACRAL SPINE

HEALTH HISTORY: GENERAL APPROACH

TECHNIQUES OF EXAMINATION

Observe the Gait

Observe the Pelvic Alignment

[Palpate the Lumbosacral Spine and Adjoining Structures](#)

[Assess Lumbosacral Spine Range of Motion](#)

[Perform Special Maneuvers on the Lumbosacral Spine Joint \(If Indicated\)](#)

ANATOMY: HIP JOINT

HEALTH HISTORY: GENERAL APPROACH

TECHNIQUES OF EXAMINATION

[Inspect the Hip Joint](#)

[Palpate the Anterior Hip Landmarks](#)

[Palpate the Posterior Hip Landmarks](#)

[Assess Hip Joint Range of Motion](#)

[Perform Special Maneuvers on the Hip Joint \(If Indicated\)](#)

ANATOMY: KNEE

HEALTH HISTORY: GENERAL APPROACH

TECHNIQUES OF EXAMINATION

[Inspect the Gait](#)

[Inspect the Knee and Surrounding Structures](#)

[Palpate the Knee and Adjoining Structures](#)

[Assess Knee Range of Motion](#)

[Perform Special Maneuvers on the Knee Joint \(If Indicated\)](#)

ANATOMY: ANKLE JOINT AND FOOT

HEALTH HISTORY: GENERAL APPROACH

[Ankle Pain or Swelling](#)

[Foot Pain](#)

TECHNIQUES OF EXAMINATION

[Inspect the Ankle and Foot](#)

[Palpate the Ankle, Foot and Surrounding Structures](#)

[Assess Ankle and Foot Range of Motion](#)

[Perform Special Maneuvers on the Ankle and Foot \(If Indicated\)](#)

SPECIAL TECHNIQUES AND MANEUVERS

[Analyzing Gait](#)

[Measuring Leg Length](#)

[Describing Limited Joint Motion](#)

[Screening for Scoliosis](#)

[Detecting Knee Joint Effusions](#)

[Modifications in Physical Examinations: Best Practices for Specialized Patient Populations](#)

RECORDING YOUR FINDINGS

POINT-OF-CARE ULTRASOUND EXAMINATION

[Detecting Joint Fluid](#)

HEALTH PROMOTION AND COUNSELING: EVIDENCE AND RECOMMENDATIONS

[Osteoporosis: Burden of Disease and Risk Factors](#)

[Osteoporosis: Screening Recommendations](#)

[Osteoporosis: Assessing Fracture Risk](#)

REFERENCES

CHAPTER 27

Nervous System

ANATOMY AND PHYSIOLOGY

Central Nervous System

Peripheral Nervous System

Motor Pathways

Spinal Reflexes: Muscle Stretch Response

Sensory Pathways

HEALTH HISTORY: GENERAL APPROACH

Headache

Dizziness or Lightheadedness

Weakness

Numbness or Abnormal or Absent Sensation

Fainting and Blacking Out (Near-Syncope and Syncope)

Seizures

Tremors or Involuntary Movements

PHYSICAL EXAMINATION: GENERAL APPROACH

TECHNIQUES OF EXAMINATION: MENTAL STATUS

Assess Mental Status

TECHNIQUES OF EXAMINATION: CRANIAL NERVES

Test for Cranial Nerve I—Olfactory

Test for Cranial Nerve II—Optic

Test for Cranial Nerves II and III—Optic and Oculomotor

Test for Cranial Nerves III, IV, and VI—Oculomotor, Trochlear, and Abducens

Test for Cranial Nerve V—Trigeminal

Test for Cranial Nerve VII—Facial

Test for Cranial Nerve VIII—Vestibulocochlear

Test for Cranial Nerves IX and X—Glossopharyngeal and Vagus

Test for Cranial Nerve XI—Spinal Accessory

Test for Cranial Nerve XII—Hypoglossal

TECHNIQUES OF EXAMINATION: MOTOR SYSTEM

Assess the Motor System for Body Position, Involuntary Movements, and Muscle Characteristics

Test Abduction at the Shoulder

Test Flexion and Extension at the Elbow

Test Flexion and Extension at the Wrist

Test Extension and Abduction of the Fingers

Test for Adduction and Abduction of the Thumb

Test Flexion and Extension at the Hips

Test Adduction and Abduction at the Hips

Test Flexion and Extension at the Knee

Test Dorsiflexion and Plantar Flexion of the Ankle

TECHNIQUES OF EXAMINATION: REFLEXES

[Elicit the Biceps Reflex \(C5, C6\)](#)
[Elicit the Triceps Reflex \(C6, C7\)](#)
[Elicit the Brachioradialis Reflex \(C5, C6\)](#)
[Elicit the Quadriceps \(Patellar\) Reflex \(L2–L4\)](#)
[Elicit the Achilles \(Ankle\) Reflex \(Primarily S1\)](#)
[Test for Clonus \(if indicated\)](#)
[Assess Plantar Responses](#)
[Elicit Superficial Stimulation Reflex](#)

TECHNIQUES OF EXAMINATION: SENSORY SYSTEM, COORDINATION, STATION, AND GAIT

[Assess Sensation](#)
[Test Coordination \(Rapid Alternating Movements\)](#)
[Test Coordination \(Point-to-Point Movements\)](#)
[Perform Gait Assessment](#)
[Assess Station \(Romberg Test\)](#)

SPECIAL TECHNIQUES AND MANEUVERS

[Detecting Meningeal Irritation](#)
[Assessing for Lumbosacral Radiculopathy](#)
[Detecting Asterixis \(Flapping Tremor\)](#)
[Assessing Scapular Winging](#)
[Evaluating Neurologic Status in Comatose Patients](#)
[Modifications in Physical Examinations: Best Practices for Specialized Patient Populations](#)

RECORDING YOUR FINDINGS

HEALTH PROMOTION AND COUNSELING: EVIDENCE AND RECOMMENDATIONS

[Preventing Cerebrovascular Disease](#)
[Aspirin Chemoprophylaxis](#)
[Screening for Atrial Fibrillation](#)
[Screening for Asymptomatic Carotid Artery Stenosis](#)

REFERENCES

UNIT 3

Special Populations

CHAPTER 28

Children: Infancy Through Adolescence

INTRODUCTION

GENERAL PRINCIPLES OF CHILD DEVELOPMENT

SURVEILLANCE OF DEVELOPMENT

KEY COMPONENTS OF HEALTH PROMOTION

NEWBORNS AND INFANTS

HEALTH HISTORY: GENERAL APPROACH

SURVEILLANCE OF DEVELOPMENT

PHYSICAL EXAMINATION: GENERAL APPROACH

Newborns

Infants

TECHNIQUES OF EXAMINATION: NEWBORNS AND INFANTS

Apgar Score

Gestational Age and Birth Weight

General Survey

Somatic Growth

Vital Signs

Skin

Head

Eyes

Ears

Nose

Mouth and Pharynx

Neck

Thorax and Lungs

Heart

Peripheral Vascular System

Breasts

Abdomen

Penis, Testes, and Scrotum

Vulva and Vagina

Rectum and Anus

Musculoskeletal System

Nervous System

RECORDING YOUR FINDINGS

HEALTH PROMOTION AND COUNSELING: EVIDENCE AND RECOMMENDATIONS

PRESCHOOL-AGE AND SCHOOL-AGE CHILDREN

HEALTH HISTORY: GENERAL APPROACH

SURVEILLANCE OF DEVELOPMENT: EARLY CHILDHOOD, 1 TO 4 YEARS

SURVEILLANCE OF DEVELOPMENT: MIDDLE CHILDHOOD, 5 TO 10 YEARS

PHYSICAL EXAMINATION: GENERAL APPROACH

TECHNIQUES OF EXAMINATION

Somatic Growth

Vital Signs

Skin

Head

Eyes

Ears

Nose and Sinuses

[Mouth and Pharynx](#)

[Neck](#)

[Thorax and Lungs](#)

[Heart and Vascular System](#)

[Abdomen](#)

[Penis and Scrotum](#)

[Vulva and Surrounding Structures](#)

[Rectum and Anus](#)

[Musculoskeletal System](#)

[Nervous System](#)

RECORDING YOUR FINDINGS

HEALTH PROMOTION AND COUNSELING: EVIDENCE AND RECOMMENDATIONS

[Children 1 to 4 Years](#)

[Children 5 to 10 Years](#)

ADOLESCENTS

HEALTH HISTORY: GENERAL APPROACH

[HEEADSSS Assessment](#)

SURVEILLANCE OF DEVELOPMENT: 11 TO 20 YEARS

[Gender and Sexual Identity Formation among Adolescents](#)

PHYSICAL EXAMINATION: GENERAL APPROACH

TECHNIQUES OF EXAMINATION

SPECIAL TECHNIQUES AND MANEUVERS

[Assessing for Scoliosis](#)

[Conducting Sports Preparticipation Physical Evaluation](#)

RECORDING YOUR FINDINGS

HEALTH PROMOTION AND COUNSELING: EVIDENCE AND RECOMMENDATIONS

REFERENCES

CHAPTER 29

Pregnant Persons

ANATOMY AND PHYSIOLOGY

[Anatomic Changes](#)

[Physiologic Hormonal Changes](#)

HEALTH HISTORY: GENERAL APPROACH

[Initial Prenatal Visit](#)

[Subsequent Prenatal Visits](#)

PHYSICAL EXAMINATION: GENERAL APPROACH

TECHNIQUES OF EXAMINATION

[Position the Pregnant Individual](#)

[Prepare the Examining Equipment](#)

[Conduct a General Inspection](#)

[Measure Height, Weight, and Vital Signs](#)

[Inspect the Head and Neck](#)

[Inspect, Percuss, and Auscultate the Thorax and Lungs](#)

[Examine the Heart](#)

[Examine the Breasts](#)

[Examine the Abdomen](#)

[Examine the External Genitalia](#)

[Inspect the Internal Genitalia](#)

[Examine the Internal Genitalia](#)

[Consider Examining the Anus, Rectum, and Rectovaginal Septum](#)

[Examine the Extremities](#)

SPECIAL TECHNIQUES

[Leopold Maneuvers](#)

RECORDING YOUR FINDINGS

HEALTH PROMOTION AND COUNSELING: EVIDENCE AND RECOMMENDATIONS

[Screening for Infectious Diseases](#)

[Genetic Testing and Aneuploidy Screening](#)

[Screening for Medical Conditions Associated with Pregnancy](#)

[Screening for Perinatal Depression](#)

[Intimate Partner Violence Screening](#)

[Rh\(D\) Incompatibility Screening](#)

[Screening and Counseling for Unhealthy Behaviors](#)

[Counseling for Healthy Weight and Weight Gain in Pregnancy](#)

[Interventions to Prevent Perinatal Depression](#)

[Breastfeeding or Chestfeeding](#)

[Immunizations in Pregnancy](#)

[Preventive Medications](#)

[Unintended Pregnancy](#)

REFERENCES

CHAPTER 30

Older Adults

ADDRESSING THE CHALLENGES OF GLOBAL AGING

DEFINING OLDER ADULTHOOD

ANATOMIC AND PHYSIOLOGIC CHANGES IN OLDER ADULTS

COMMUNICATING EFFECTIVELY WITH OLDER ADULTS

GERIATRIC 5Ms

HEALTH HISTORY: APPLYING THE 5Ms FRAMEWORK

[Mind](#)

[Mobility](#)

[Medications](#)

[Multicomplexity](#)
[Matters Most](#)

ADJUSTMENTS IN PHYSICAL EXAMINATION TECHNIQUES FOR OLDER ADULTS

GERIATRIC ASSESSMENT: APPLYING THE 5Ms FRAMEWORK

[Mind](#)
[Mobility](#)
[Medications](#)
[Multicomplexity](#)
[Matters Most](#)

HEALTH PROMOTION AND COUNSELING: EVIDENCE AND RECOMMENDATIONS

[When to Screen](#)
[Immunizations](#)
[Cancer Screening](#)
[Elder Abuse](#)
[Household Safety and Falls Prevention](#)

REFERENCES

[Index](#)

List of Tables

CHAPTER 6 Clinical Documentation and Oral Presentation

Table 6-1: Special Considerations in Clinical Documentation

CHAPTER 11 Cognition, Behavior, and Mental Status

Table 11-1: Central Nervous System Structures and Mental Disorders

Table 11-2: Neurocircuitry of Mental Disorders

Table 11-3: Neurocognitive Disorders: Delirium and Dementia

Table 11-4: Somatic Symptom and Related Disorders

Table 11-5: Screening for Depression: Geriatric Depression Scale (Short Form)

Table 11-6: Screening for Depression: Patient Health Questionnaire (PHQ-9)

Table 11-7: Screening for Anxiety Disorders: GAD-7

Table 11-8: Screening for Dementia: Mini-Cog

Table 11-9: Screening for Dementia: Montreal Cognitive Assessment (MoCA)

Table 11-10: Screening for Cognitive Impairment and Dementia: Saint Louis University Mental Status Exam (SLUMS)

CHAPTER 12 Skin, Hair, and Nails

Table 12-1: Describing Primary Skin Lesions: Flat, Raised, and Fluid-Filled

Table 12-2: Additional Primary Lesions: Pustules, Furuncles, Nodules, Cysts, Wheals, Burrows

Table 12-3: Dermatology Safari: Benign Lesions

Table 12-4: Rough Lesions: Actinic Keratoses, Squamous Cell Carcinoma, and their Mimics

Table 12-5: Pink Lesions: Basal Cell Carcinoma and Its Mimics

Table 12-6: Brown Lesions: Melanoma and Its Mimics

Table 12-7: Vascular and Purpuric Lesions of the Skin

Table 12-8: Hair Loss

Table 12-9: Findings in or Near the Nails

Table 12-10: Systemic Diseases and Associated Skin Findings

Table 12-11: Acne Vulgaris—Primary and Secondary Lesions

Table 12-12: Signs of Sun Damage

Table 12-13: Pressure Injuries

CHAPTER **13** Head and Neck

Table 13-1: Symptoms and Signs of Thyroid Dysfunction

Table 13-2: Thyroid Enlargement and Function

CHAPTER **14** Eyes

Table 14-1: Red Eyes

Table 14-2: Visual Field Defects

Table 14-3: Variations and Abnormalities of the Eyelids

Table 14-4: Lumps and Swellings in and Around the Eyes

Table 14-5: Opacities of the Cornea and Lens

Table 14-6: Pupillary Abnormalities

Table 14-7: Dysconjugate Gaze

Table 14-8: Normal Variations of the Optic Disc

Table 14-9: Abnormalities of the Optic Disc

Table 14-10: Retinal Arteries and Arteriovenous Crossings: Normal and Hypertensive

Table 14-11: Red Spots and Streaks in the Fundi

Table 14-12: Light-Colored Spots in the Fundi

CHAPTER **15** Ears and Nose

Table 15-1: Dizziness and Vertigo

Table 15-2: Lumps on or Near the Ear

Table 15-3: Abnormalities of the Tympanic Membrane

Table 15-4: Patterns of Hearing Loss

CHAPTER **16** Throat and Oral Cavity

Table 16-1: Abnormalities of the Lips

Table 16-2: Findings in the Pharynx, Palate, and Oral Mucosa

Table 16-3: Findings in the Gums and Teeth

Table 16-4: Findings in or under the Tongue

CHAPTER **17** Thorax and Lungs

Table 17-1: Normal and Altered Breath and Voice Sounds

Table 17-2: Dyspnea

Table 17-3: Cough and Hemoptysis

Table 17-4: Chest Pain

Table 17-5: Abnormalities in Rate and Rhythm of Breathing

Table 17-6: Deformities of the Thorax

Table 17-7: Adventitious (Added) Lung Sounds: Causes and Qualities

Table 17-8: Physical Findings in Selected Chest Disorders

CHAPTER **18** Cardiovascular System

Table 18-1: Selected Heart Rates and Rhythms

Table 18-2: Selected Irregular Rhythms

Table 18-3: Abnormalities of the Arterial Pulse and Pressure Waves

Table 18-4: Variations and Abnormalities of the Ventricular Impulses

Table 18-5: Variations in the First Heart Sound—S1

Table 18-6: Variations in the Second Heart Sound—S2

Table 18-7: Extra Heart Sounds in Systole

Table 18-8: Extra Heart Sounds in Diastole

Table 18-9: Midsystolic Murmurs

Table 18-10: Pansystolic (Holosystolic) Murmurs

Table 18-11: Diastolic Murmurs

Table 18-12: Cardiovascular Sounds with Both Systolic and Diastolic Components

CHAPTER **19** Peripheral Vascular System

Table 19-1: Types of Peripheral Edema

Table 19-2: Painful Peripheral Vascular Disorders and Their Mimics

Table 19-3: Chronic Insufficiency of Arteries and Veins

Table 19-4: Common Ulcers of the Ankles and Feet

CHAPTER **20** Breasts and Axillae

Table 20-1: Common Breast Masses

Table 20-2: Visible Signs of Breast Cancer

CHAPTER **21** Abdomen

Table 21-1: Classification and Diagnosis of Selected Functional Gastrointestinal Disorders

Table 21-2: Abdominal Pain

Table 21-3: Dysphagia

Table 21-4: Diarrhea

Table 21-5: Constipation

Table 21-6: Black and Bloody Stool

Table 21-7: Localized Bulges in the Abdominal Wall

Table 21-8: Protuberant Abdomens

Table 21-9: Sounds in the Abdomen

Table 21-10: Tender Abdomens

Table 21-11: Liver Enlargement: Apparent and Real

CHAPTER **22** Anus and Rectum

Table 22-1: Abnormalities of the Anus, Surrounding Skin, and Rectum

CHAPTER **23** Pelvis and Genitourinary System: Penis, Scrotum, and Prostate

- Table 23-1:** Urinary Frequency, Nocturia, and Polyuria
- Table 23-2:** Urinary Incontinence in Individuals Assigned Male at Birth
- Table 23-3:** Sexually Transmitted Infection–Associated Changes in Genital Anatomy
- Table 23-4:** Abnormalities of the Penis and Scrotum
- Table 23-5:** Abnormalities of the Testis
- Table 23-6:** Abnormalities of the Epididymis and Spermatic Cord
- Table 23-7:** Course, Presentation, and Differentiation of Hernias in the Groin
- Table 23-8:** BPH Symptom Score: American Urological Association
- Table 23-9:** Abnormalities of the Prostate

CHAPTER **24** Pelvis and Genitourinary System: Vulva, Vagina, Uterus, and Adnexa

- Table 24-1:** Bulges and Swelling of the Vulva, Vagina, and Urethra
- Table 24-2:** Lesions of the Vulva
- Table 24-3:** Vaginal Discharge
- Table 24-4:** Urinary Incontinence in Individuals Assigned Female at Birth
- Table 24-5:** Variations in the Cervical Surface
- Table 24-6:** Shapes of the Cervical Os
- Table 24-7:** Abnormalities of the Cervix
- Table 24-8:** Positions of the Uterus Retroversion and Retroflexion Are Usually Normal Variants
- Table 24-9:** Abnormalities of the Uterus
- Table 24-10:** Adnexal Masses

CHAPTER **25** Musculoskeletal System: Neck, Shoulders, and Upper Extremities

- Table 25-1:** Patterns of Pain in and Around the Joints
- Table 25-2:** Systemic Manifestations of Musculoskeletal Disorders
- Table 25-3:** Pains in the Neck
- Table 25-4:** Painful Shoulders
- Table 25-5:** Swollen or Tender Elbows
- Table 25-6:** Arthritis in the Hands
- Table 25-7:** Swellings and Deformities of the Hands
- Table 25-8:** Tendon Sheath, Palmar Space, and Finger Infections

CHAPTER **26** Musculoskeletal System: Lumbosacral Spine, Hips, and Lower Extremities

- Table 26-1:** Low Back Pain
- Table 26-2:** Abnormalities of the Feet
- Table 26-3:** Abnormalities of the Toes and Soles

CHAPTER **27** **Nervous System**

Table 27-1: Disorders of the Central and Peripheral Nervous Systems

Table 27-2: Disorders of Speech

Table 27-3: Abnormalities of Gait and Posture

Table 27-4: Primary Headaches

Table 27-5: Secondary Headaches and Cranial Neuralgias

Table 27-6: Syncope and Similar Disorders

Table 27-7: Types of Stroke

Table 27-8: Seizure Disorders

Table 27-9: Tremors and Involuntary Movements

Table 27-10: Nystagmus

Table 27-11: Types of Facial Paralysis

Table 27-12: Abnormal Body Postures

Table 27-13: Disorders of Muscle Tone

Table 27-14: Glasgow Coma Scale

Table 27-15: Metabolic and Structural Coma

Table 27-16: Pupils in Comatose Patients

CHAPTER **28** **Children: Infancy Through Adolescence**

Table 28-1: Abnormalities in Heart Rhythm

Table 28-2: Abnormalities in Blood Pressure

Table 28-3: Common Skin Rashes and Skin Findings in Newborns and Infants

Table 28-4: Warts and Lesions that Resemble Warts, and Other Raised Lesions

Table 28-5: Common Skin Lesions during Childhood

Table 28-6: Abnormalities of the Head

Table 28-7: Diagnostic Facies in Infancy and Childhood

Table 28-8: Abnormalities of the Eyes, Ears, and Mouth

Table 28-9: Abnormalities of the Teeth, Pharynx, and Neck

Table 28-10: Cyanosis in Children

Table 28-11: Congenital Heart Murmurs

Table 28-12: Common Abnormalities in the Genitourinary Anatomy Associated with Testes and Penises

Table 28-13: Common Musculoskeletal Findings in Young Children

Table 28-14: Power of Prevention: Vaccine-Preventable Diseases

CHAPTER **29** **Pregnant Persons**

Table 29-1: Normal Anatomic and Physiologic Changes in Pregnancy

UNIT 1

Foundations of Health Assessment

CHAPTER 1

Foundational Skills Essential to the Clinical Encounter

“The ritual of one individual coming to another and telling [them] things that [they] would not tell [their] preacher or rabbi; and then, incredibly, on top of that, disrobing and allowing touch ... I think our skills in examining a patient have to be worthy of that kind of trust.”

—Abraham Verghese, MD, A Doctor's Touch, TEDGlobal, 2011.

THE CLINICAL ENCOUNTER

As you begin your clinical training, you'll learn a range of essential skills that will deepen your patient relationships and enhance your ability to provide care. A *clinical skill* is any discrete act within the overall process of patient care. ¹ Clinical skills are the singular elements that constitute clinical competence. Through the purposeful selection and integration of these individual skillful acts during the patient encounter, you will lay the foundation for high-quality clinical care.

Your clinical skills will evolve over time as you develop your professional relationship with each patient, take their clinical history, perform a mental and physical examination, conduct tests and procedures, and provide diagnostic and therapeutic interventions. ² To become a skilled clinician, you will need to integrate contemporary biomedical knowledge into your patient care in a professional and culturally sensitive manner. ² As a student, you will gradually move from passive observation to active patient assessment, gaining confidence and expertise with each encounter. You will also need to commit to ongoing practice and honest self-assessment in order to continuously improve your skills.

The initial chapters in this unit will introduce you to the essentials of the clinical encounter, with a focus on establishing trust, which is the foundation of the therapeutic alliance with patients (Fig. 1-1). Initially, you will focus on gathering information, but with experience and empathic listening, you will learn to allow the patient's story to unfold in its most authentic and detailed form. By mastering these skills and building caring patient relationships based on mutual trust and respect, you will reap the timeless rewards of the clinical professions. These are the fundamental features of all clinical care. ²



FIGURE 1-1 . Therapeutic alliance between clinician and patient.

Chapter Content Guide

- Approach to the Clinical Encounter
- Structure and Sequence of the Clinical Encounter
- Disparities in Health Care
- Other Major Considerations

APPROACH TO THE CLINICAL ENCOUNTER

The clinical encounter involves both the clinician and the patient and can be approached in two ways: *clinician-centered* and *patient-centered*. The former approach prioritizes acquiring details about symptoms and disease, potentially neglecting the personal dimensions of illness.^{3, 4} In contrast, the patient-centered approach values patients' expressions of personal concerns and emotions and considers the patient's perspective of illness.

The *disease/illness distinction model* helps to clarify the distinct but complementary perspectives of the clinician and the patient (Box 1-1).⁵ A successful clinical interview must incorporate both the clinician's and the patient's perspectives of reality, disease, and illness.³

Research indicates that integrating both clinician-centered and patient-centered approaches in a clinical encounter leads to a more comprehensive understanding of the patient's illness and enables clinicians to convey qualities of respect, empathy, humility, and sensitivity.^{3, 7} This merged

approach has been shown to be more satisfying for both patients and clinicians and more effective in achieving desired health outcomes.^{8, 9} By incorporating both perspectives in your clinical encounters, you can gain a more well-rounded view of the patient's problems from both your own and their point of view. Achieving a balance between these two essential components is critical for conducting an effective clinical interview in a patient encounter.

Box 1-1. Disease/Illness Distinction Model

Disease	Explanation used by the clinician to organize symptoms, leading to a clinical diagnosis
Illness	Construct that explains how the patient experiences the disease, including its impact on relationships, function, and overall well-being

For example, when a patient presents with a sore throat, the clinician may focus on identifying the cause of the symptoms, such as streptococcal pharyngitis or allergies to penicillin. However, the patient's concerns may extend beyond the physical symptoms to include worries about pain, difficulty swallowing, missing work, or even the fear of developing cancer.

These divergent concerns illustrate the importance of incorporating both the clinician's and the patient's perspectives into the clinical interview.^{3, 6}

STRUCTURE AND SEQUENCE OF THE CLINICAL ENCOUNTER

In general, an effective clinical encounter moves through a logical sequence (Box 1-2).¹⁰ In this chapter, we focus on the behaviors related to the initiation and closure of the clinical encounter as well as the exploration of the patient's perspectives of their illness. An illustrative example of this framework is the enhanced *Calgary–Cambridge Guides* (Fig. 1-2), which describes the structure and timeline of the clinical encounter and highlight the need to elicit information regarding both the biomedical disease process and the patient's perspective. They also include a place for the physical examination. The structure includes five major steps: *initiating the session*, *information gathering*, *physical examination*, *explaining and planning*, and *closing the session*.^{11–13}

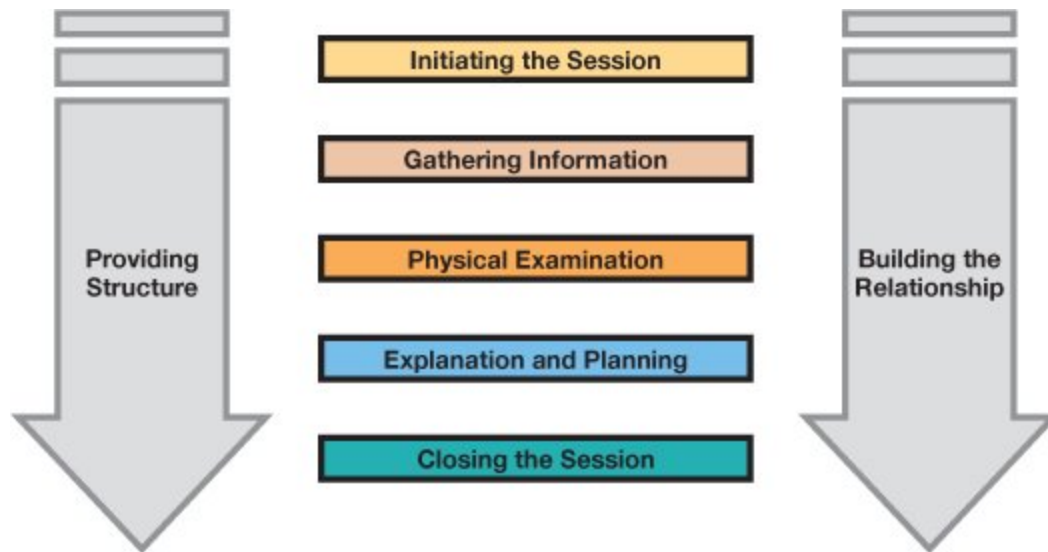


FIGURE 1-2 . Enhanced Calgary–Cambridge Guides: Structure and timeline of the clinical encounter. (Reproduced with permission from Kurtz S, Silverman J, Benson J, Draper J. Marrying content and process in clinical method teaching: enhancing the Calgary–Cambridge guides. *Acad Med* . 2003;78[8]:802–809.)

Box 1-2. General Structure and Sequence of the Clinical Encounter

1. Initiating the encounter
 - Setting the stage/preparation
 - Greeting the patient and establishing initial rapport
2. Gathering information
 - Initiating information gathering
 - Exploring patient's perspective of illness
 - Exploring biomedical perspective of disease including relevant background and context
3. Performing the physical examination
4. Explaining and planning
 - Provide correct amount and type of information
 - Negotiate plan of action
 - Shared decision making
5. Closing the encounter

Note: Two additional frameworks occur as continuous threads throughout the sequence—namely, building the relationship and structuring the interview (see Fig. 1-2).

Source: Adapted from Kurtz S, Silverman J, Benson J, Draper J. Marrying content and process in clinical method teaching: enhancing the Calgary–Cambridge guides. *Acad Med* . 2003;78(8):802–809; van de Poel K, Vanagt E, Schrimpf U, Gasiorek J. *Communication Skills for Foreign and Mobile Medical Professionals* . Springer; 2013:xvii, 145; de Haes H, Bensing J. Endpoints in medical communication research, proposing a framework of functions and outcomes. *Patient Educ Couns* . 2009;74(3):287–294.

Stage 1: Initiating the Encounter

This is the stage of relationship-building with your patient. Fostering the patient–clinician relationship is critical because without a good relationship, none of the other goals of the clinical encounter can be pursued in an optimal manner.¹⁴ Respect, trust, and rapport are necessary components of a burgeoning therapeutic relationship.

Set the Stage. To start, set the stage and prepare for the interview. Check your appearance, make sure the patient is comfortable, and create an environment that is conducive to sharing personal information. Each interview has its own rhythm and sequence, so mastering the steps described is essential. Also, reflect on any biases you may have that could color your reactions to the patient and the therapeutic alliance you need to create. See *Racism and Bias*, p. 11.

Adjust the Environment. Adjust the environment to ensure effective communication with your patient. Even in challenging settings like a two-bedded hospital room or a busy emergency department, improving communication is worth the effort. Use privacy curtains or move to an empty room to create a more confidential setting.

Consider factors like cultural background and individual preferences about interpersonal space. Choose a distance that allows for good eye contact and clear conversation, pull up a chair, and sit at eye level with the patient to facilitate communication. Move any physical barriers out of the way to create an open and welcoming environment (Fig. 1-3).



FIGURE 1-3 . Move physical barriers out of the way and be at eye level.

Be mindful of arrangements that may convey disrespect, such as interviewing a patient who is already positioned for a pelvic exam or talking through a bathroom door. Ensure that any computer monitors are positioned so as not to obstruct your view of the patient or hide your face from them.

Lighting is another essential consideration. Create a private and welcoming environment to help facilitate effective communication and build a positive rapport with your patient.

Review the Clinical Record. Before seeing the patient, review the clinical record (Fig. 1-4). It will provide important background information and suggest areas to explore during the visit. Review identifying data such as age, gender, address, and insurance as well as the Patient Problem List, medications, and allergies.



FIGURE 1-4 . Review the health record before the clinical encounter.

The clinical record usually contains past diagnoses and treatments, but making your assessment based on what you learn during the visit is important. Keep in mind that the clinical record is compiled from many observers, and data may be incomplete or even contradict what the patient tells you. Reconcile any discrepancies in the record for the patient's care. Be aware of potential problems that may arise from documentation mismatches, especially as electronic health records are being developed to include preferred names and gender pronouns. Address any issues related to this or other sensitive topics that may arise during the visit. Being thorough in your review of the clinical record allows you to provide the best possible care for your patient.

Set Your Agenda. Before speaking with the patient, clarify your goals for the interview. A student goal may be completing a comprehensive history, whereas for an advanced trainee or clinician, goals range from assessing a new concern to treatment follow-up. Regardless, balancing clinician-centered and patient-centered goals is essential, as is considering the agendas of the patient, their family, and health care agencies. Taking a few minutes to think about your goals will help you align your priorities with the patient's agenda. ¹⁵

Greet the Patient and Establish Initial Rapport. The initial moments of your encounter lay the foundation for your ongoing relationship. How you greet the patient and other visitors in the room, provide for the patient's comfort, and arrange the physical setting, all shape the patient's first impressions (Fig. 1-5). Relating effectively with patients is among the most valued skills of clinical care. For the patient, "a feeling of connectedness ... of being deeply heard and understood ... is the very heart of healing."¹⁶ For the clinician, this deeper relationship enriches the rewards of patient care.^{17 – 19} Suggestions on how to establish rapport with specific patient populations are shown in Box 1-3 .



FIGURE 1-5 . Greeting the patient and establishing rapport.

As you begin, *welcome the patient by introducing yourself*, giving your own first and last name. If possible, shake hands with the patient. If this is the first time you are seeing the patient, explain your role, your status as a student or trainee, and how you will be involved in their care.

Identify Preferred Title, Name and Pronouns. As much as possible, let the patient dictate how they would like to be addressed (Box 1-6). Clinicians should ask all patients their *preferred titles* , *preferred name* , and *pronouns* , ideally at the beginning of the visit and/or on an intake questionnaire.

Preferred Title. Titles include Mr., Mrs., Ms., or honorifics such as Professor or Doctor. This not only provides valuable information about the patient's identity but is also important in establishing rapport and showing respect, especially if you are seeing the patient for the first time and promotes a welcoming environment.

Preferred Name. The name may be a nickname (e.g., "Pat" for "Patrick"), use of a middle name, or some other name altogether. After stating your name, ask the patient what name they would like you to use. Except with children or adolescents, avoid first names until you have specific permission. Calling a patient "*dear*," "*sweetie*," or overly familiar names can depersonalize and demean them.²¹

If you are unsure how to pronounce the patient's name, do not be afraid to ask. You can say, “ *I am afraid of mispronouncing your name. Could you say it for me?* ” Then repeat it to make sure you heard it correctly. For patients who identify as transgender and gender nonbinary, the preferred name may match their affirmed gender and be different than their given name, which may have matched their sex assigned at birth. ²⁰

Pronouns. When asking patients about their pronouns, it can be helpful to share your own pronouns with patients, asking: “*Which gender pronouns do you use?*” (Box 1-7). For example, “*I use . . . he and him/she and hers/they and theirs.*” Some of your patients may use *gender-neutral pronouns* or *inclusive pronouns* . These are pronouns that are not associated with a particular gender. They are used by individuals who do not identify as strictly male or female or who prefer not to be referred to as he/him or she/her. Some examples include *they/them*, *ze/hir*, *xe/xem*, and *ey/em* . These pronouns are used to promote inclusivity and respect for individuals who identify outside of the gender binary.

To show respect and avoid making patients feel disrespected or invalidated, it's crucial to use the correct title, name, and pronoun they have provided. Misgendering a patient can lead to feelings of alienation, dysphoria, or disrespect. **While it's not always possible to avoid mistakes, a simple apology can go a long way.** Instead of over-apologizing, say something like, “*I apologize for using the wrong pronoun [or preferred name]. I did not mean to disrespect you.*” It can be tempting to overstate how badly you feel about making a mistake, but that might make the misgendered patient feel more awkward and inclined to comfort you, which is not appropriate.

Box 1-3. Approach to Establishing Rapport with Specific Populations

Population	Recommendations
Newborns and Infants (birth to 30 days; 1 month to 1 year)	■ Building rapport is still important even though newborns and infants are unable to communicate like older children. ■ Congratulate the family on their newborn, encourage caregivers to feed the newborn either while talking or before the encounter to keep the newborn calm, and focus on the caregivers to ask about their well-being. ■ Take advantage of opportunities to obtain quick screening questions about family health topics.
Young and School-Age Children (1 to 4 years; 5 to 10 years) ^{22–25}	■ To begin the encounter with a child patient and their family, introduce yourself first, then engage the child with play to build rapport and manage their mood. ■ While the child is playing, obtain the health history from the caregiver, asking for confirmation or elaboration as necessary. ■ For school-age children, familiarize yourself with “kid culture” and ask age-appropriate questions to facilitate engagement.
Adolescents ^{26–28}	■ When interacting with adolescent patients, prioritize their confidentiality and trust while ensuring family members and caregivers feel comfortable and heard. ■ Use open-ended questions, provide ample opportunities for adolescents to share their concerns, and acknowledge the importance of their privacy. ■ Inform the family that they will have an opportunity to speak with you but first spend time alone with the adolescent patient without any family members present.
Older Adults ²¹	■ When interacting with older adult patients, elicit their preferred way of being addressed and adjust the environment to put them at ease.



- Provide a well-lit, moderately warm setting with minimal background noise, chairs with arms, and ample space for safe navigation, particularly if using an assistive device.
- Allow time for open-ended questions, reminiscing, and include family and caregivers, especially when cognitive impairment is present.

Patients with Physical and Sensory Disabilities



- When referring to patients with disabilities, use “people-first” language (e.g., *person who is blind*, *person who uses a wheelchair*, *person with hearing loss*) unless they specify otherwise.
- Always presume that patients with physical and/or sensory disabilities are competent to manage their medical care and avoid making assumptions about what assistance they may require.
- Speak directly with the patient, not to an aide or companion, and refrain from asking whether they are accompanied if they entered the room alone (Box 1-4).

Lesbian, Gay, Bisexual, Transgender, Queer + (LGBTQ+) Adults ^{29–32}



- When working with LGBTQ+ patients, recognize that they may experience anxiety related to acceptance and discomfort in disclosing their sexual behaviors and identity.
- Expand your knowledge and clinical skills in gay, lesbian, and transgender health and engage with available resources.
- Prepare to answer questions about fertility and transgender issues such as hormonal therapy and gender-affirming procedures and be open to discussing these topics with patients (Box 1-5).

Box 1-4. Establishing Rapport with Patients with Physical and Sensory Disabilities

- Based on 2023 global population estimates, more than 1.3 billion people (16% of the world's population) are estimated to live with some form of disability. ⁸⁹
- In the United States, the overall rate of people with disabilities in 2021 was estimated as 13.5% of the population. ⁹⁰

Patients Who Are Blind or Have Low Vision

- Always verbally identify yourself when you approach and introduce other people in the room.
- Do not leave without letting the patient know.
- Ask before you help. Always ask how the patient would like to be assisted.
- Be prepared to provide written materials in an auditory, tactile, or electronic format of the patient's preference (audio file, Braille, large print).
- Explain what is about to happen before beginning the encounter and ask if the patient has any questions.
- Tell the patient where personal effects (clothes and other belongings) are in the room and do not move them without telling the patient.
- Staff should be welcoming and describe the physical environment (doors, steps, ramps, bathroom location, etc.).
- Never distract or touch a service animal without asking the owner.

Patients Who Are Hard of Hearing

- Ask how best to communicate.
- Be prepared to give written materials as long as they are not the primary form of communication.
- Inform patients that sign language interpreting and real-time captioning services are available.
- If requested, promptly provide sign language interpreting or real-time captioning service for effective communication.
- Do not talk at a distance from the patient or from another room.
- Look directly at the patient when speaking so your mouth is visible.
- Speak normally and clearly. Do not shout, exaggerate mouth movements, or speak rapidly.
- Minimize background noise and glare.

Patients Who Are Deaf

- Ask how best to communicate.
- Inform patients that sign language interpreting and real-time captioning services are available.
- If requested, promptly provide sign language interpreting or real-time captioning service for effective communication.
- Do not use family members to interpret.
- Address the patient, not the interpreter.
- Be prepared to give written materials as long as they are not the primary form of communication.

Patients Who Use Wheelchairs

- Make sure there is a path of access to the room.
- Respect personal space, including wheelchair and assistive devices.

- Do not propel the wheelchair unless asked to do so.
- Provide accessible equipment as needed.
- Assist as needed, such as by clearing obstacles from the path of travel or helping patients transfer to equipment if accessible equipment is unavailable.
- Do not separate patients from their wheelchairs.

Source: Reprinted with permission from Access to Medical Care: Adults with Physical Disabilities (Project Director: Marsha Saxton, PhD). World Institute on Disability; 2016. Accessed June 16, 2023. <https://worldinstituteondisabilityblog.files.wordpress.com/2016/01/access-to-medical-care-curriculum-pdf-format.pdf>

Box 1-5. Lesbian, Gay, Bisexual, and Transgender Health

Several recent surveys provide some of the first national data sets on the lesbian, gay, bisexual, and transgender (LGBT) population.

- For the first time, in 2013, the National Health Interview Survey included a measure of sexual orientation: in the 2018 report, a sample of around 27,000 adults, 1.6% identified as gay or lesbian, 1.0% identified as bisexual, and 1.6% responded either other or did not know. Most gay and lesbian respondents were ages 18 to 44 years, with a higher percentage of bisexual respondents ages 18 to 44 years.⁹¹
- In 2012, the Gallup Daily Tracking Survey initiated the largest single study of the distribution of the LGBT population in the United States.^{92, 93} The Survey added an LGBT identity question that generated 120,000 responses: 3.4% answered “yes” when asked if they identify as LGBT. Of those identifying as LGBT, 53% were female and 6.4% were ages 18 to 29 years. Nearly 13% were in a domestic partnership or living with a partner. Non-Whites were more likely to identify as LGBT: African American 4.6%, Asians 4.3%, Hispanics 4.9%, and non-Hispanic White 3.2%.
- The 2013 American Community Survey of the United States Census Bureau reported more than 726,000 same-sex couple households; 34% had same-sex spouses.⁹⁴
- In its 2011 report on LGBT health disparities, the Institute of Medicine called for better measures of health care disparities among the diverse LGBT subpopulations to elucidate their differing health behaviors and health care needs.⁹⁵
- LGBT patients have higher rates of depression, suicide, anxiety, drug use, sexual victimization, and risk of infection with HIV and sexually transmitted infections.^{96, 97}
- One-third (33%) of transgender individuals who saw a health care provider in the past year reported having at least one negative experience related to being transgender, such as “*being refused treatment, verbally harassed, or physically or sexually assaulted, or having to teach the provider about transgender people in order to get appropriate care, with higher rates for people of color and people with disabilities.*”⁹⁸
- The Institute of Medicine has stated that barriers to accessing quality health care for LGBT adults are “a lack of providers who are knowledgeable about LGBT health needs as well as a fear of discrimination in health care settings.”⁹⁵

Box 1-6. Obtaining Patients’ Preferred Method of Address

Example:

Student: *"Good morning. I am Susannah Velasquez, a third-year clinical student. I am part of the clinical team taking care of you. I'm here to help make sure we're meeting your needs. Are you Richard Clarkson?"*

Patient: *"Yes, that's me."*

Student: *"Thank you. How would you like me to address you?"*

Patient: *"You can call me Mr. Clarkson." Or, "Richard is fine."*

Box 1-7. Obtaining Patients' Gender Pronouns

Example:

Student: *"Good morning. I am Susannah Velasquez, a third-year clinical student. I am part of the clinical team taking care of you and will assist them figure out how we can best help you. Are you Richard Clarkson?"*

Patient: *"Yes."*

Student: *"How would you like me to address you?"*

Patient: *"You can call me Mr. Clarkson." Or, "Richard is fine."*

Student: *"It is a pleasure to meet you, Mr. Clarkson. Please feel free to call me Susie. May I ask you a few more background questions before we start?"*

Patient: *"Sure."*

Student: *"In our effort to promote an inclusive and respectful environment, we like to use each other's correct pronouns. The pronouns I prefer when others refer to me are 'she' and 'her'. How about you? What pronouns do you prefer?"*

Patient: *"I use 'he' and 'him,' I guess."*

Box 1-8. Steps in Stage 2: Gathering Information

Steps	Description
Initiate information gathering	After establishing rapport, ask the patient about their chief complaint or presenting problem(s), taking notes during the interview to ensure nothing is missed (Box 1-9).
Establish the agenda for the patient encounter ³³	Ask open-ended questions to encourage patients to discuss all of their concerns, not just clinical ones. You should identify all concerns and ask about missed ones to prioritize them.
Invite the patient's story ³³	Encourage patients to share their stories in their own words using an open-ended approach, actively listening without interrupting or injecting new information to prevent bias.
Gather information about the patient's perspective of illness	Explore the patient's perspective on their illness by using different types of questions to uncover their feelings, ideas, the effect on their function, and expectations (Box 1-10).
Identify and respond to the patient's emotional cues ^{34 , 35}	Check for emotional cues and feelings by asking about them, as visits tend to be longer when you miss emotional cues. See Box 1-11 for suggested techniques.
Gather information by exploring the biomedical perspective	The health history format is a structured framework for gathering and organizing patient information in written or verbal form. It focuses one's attention on the specific kinds of information clinicians need to obtain, facilitates clinical reasoning, and standardizes communication with other health care providers.
Gather important background information and context ³⁶	Learn about the patient's life circumstances, emotional health, perception of health care, health behaviors, and access to and utilization of health care to strengthen the therapeutic alliance and improve health outcomes.

Box 1-9. A Note about Taking Notes

- Jot down or type short phrases, specific dates, or words; but do not let note-taking or the keyboard and computer screen distract you from the patient.
- Maintain good eye contact (Fig. 1-6). If the patient is talking about sensitive or disturbing material, put down your pen or move away from the keyboard.
- For patients who find note taking uncomfortable, explore their concerns and explain your need to make an accurate record.
- When using an electronic health record, face the patient directly as you elicit the patient's story, maintaining good eye contact and observing nonverbal behaviors; turn to the screen only after engaging the patient in the goals for the visit.
- Look up at the patient as often as possible, readjusting your screen and position if needed. ⁹⁹



FIGURE 1-6 . Maintain good eye contact.

Stage 2: Gathering Information

This stage has two functions: *gathering* and *providing information* (Box 1-8). ¹⁴ Clinicians gather information from patients regarding symptoms, experiences, and expectations to establish a diagnosis and treatment plan. Simultaneously, patients require information to comprehend their health issues, mitigate uncertainties, and support their coping mechanisms. This phase lays the groundwork for shared decision making later in the clinical encounter.

To explore the patient's perspective, use different types of questions. A mnemonic for the patient's perspective on the illness is *FIFE* — *F*eelings, *I*deas, *e*ffect on *F*unction, and *E*xpectations (Box

1-12). The combination of concerns and expectations has been shown to have a major influence on the patient's decision to seek help from a clinician.

Stage 3: Performing the Physical Examination

The physical examination also enhances your relationship with the patient. Physical findings denote the presence or absence of disease and an opportunity for you to learn more about your patient's outlook and condition. Since the physical examination almost always follows the history, it provides an avenue for the patient to talk about deeper fears or more serious issues. Maintain your patient's comfort throughout, avoid embarrassment, and demonstrate facility with the skills of physical examination to enhance the patient's satisfaction with the clinical encounter.³⁷

Box 1-10. Clues to the Patient's Perspective on Illness

- Direct statement(s) by the patient of explanations, emotions, expectations, and effects of the illness
- Expression of feelings about the illness without naming the illness
- Attempts to explain or understand symptoms
- Speech clues (e.g., repetition, prolonged reflective pauses)
- Sharing a personal story
- Behavioral clues indicative of unidentified concerns, dissatisfaction, or unmet needs such as reluctance to accept recommendations, seeking a second opinion, or early return appointment

Source: Lang F, Floyd MR, Beine KL. Clues to patients' explanations and concerns about their illnesses. A call for active listening. *Arch Fam Med* . 2000;9(3):222–227.

Box 1-11. Responding to Emotional Cues Using NURSE Statements^{100, 101}

Learn to respond attentively to emotional cues using techniques like reflection, feedback, and “continuers” that convey support. A mnemonic for responding to emotional cues is NURSE:

N ame: *“That sounds like a scary experience”*

U nderstand or legitimize: *“It's understandable that you feel that way”*

R espect: *“You've done better than most people would with this”*

S upport: *“I will continue to work with you on this”*

E xplore: *“How else were you feeling about it?”*

Source: Communication: what do patients want and need? *J Oncol Pract* . 2008;4(5):249–253; Pollak KI, Arnold RM, Jeffreys AS, et al. Oncologist communication about emotion during visits with patients with advanced cancer. *J Clin Oncol* . 2007;25(36):5748–5752.

Stage 4: Explaining and Planning

This stage includes the elaboration of the patient's chief concerns from the disease and illness perspectives. Your goal is to assess and respond to the patient's needs for information. To achieve a shared understanding, make it easy for the patient to understand and remember your explanations and

encourage mutual discussion rather than one-way communication. This will allow your patients to understand shared clinical decision making, determine how much they want to be involved, and hopefully increase their commitment to the plans made.

Box 1-12. Exploring the Patient's Perspective (F-I-F-E) ^{3, 6}

- The patient's **F**eelings, including fears or concerns, about the problem
- The patient's **I**deas about the nature and the cause of the problem
- The effect of the problem on the patient's life and **F**unction
- The patient's **E**xpectations of the disease, of the clinician, or of health care, often based on prior personal or family experiences

Source: Fortin AH VI, Dwamena FC, Frankel RM, Smith RC. *Smith's Patient-Centered Interviewing: An Evidence-Based Method*. 3rd ed. McGraw-Hill Medical; 2012; Mauksch L, Farber S, Greer HT. Design, dissemination, and evaluation of an advanced communication elective at seven U.S. medical schools. *Acad Med*. 2013;88(6):843–851.

Provide Useful Information and Verify Patient Understanding. ^{38 – 40} Studies show that patients forget up to 80% of the clinical information provided during office visits, with nearly half of the information retained being incorrect. To verify the patient's understanding of the plan of care, the “teach-back” technique can be used, in which the patient is asked to explain the plan of care in their own words. The “show-me” method can also be used to confirm that patients can follow specific instructions (Box 1-13).

Negotiate the Plan of Action through Shared Decision Making. ^{41, 42} Interactive history taking creates a shared understanding of the patient's issues, serving as a basis for further evaluation and treatment planning. *Shared decision making* involves a three-step process of introducing choices, exploring patient preferences, and moving to a decision while checking for readiness and offering additional time if needed. This approach promotes optimal therapy, treatment adherence, and patient satisfaction. It may be necessary to explain recommendations multiple times to ensure the patient understands and agrees with the proposed plan.

Stage 5: Closing the Encounter

Ending the interview or visit can be challenging since patients often have numerous questions and feel engaged if the clinician has done their job well. Inform the patient of the approaching end to allow for any final questions, but avoid bringing up a new topic during the last few minutes, unless it concerns a life-threatening issue. Instead, assure the patient of your interest and arrange to address the issue at a future time. Reaffirming your commitment to the patient's health shows involvement and esteem. Summarizing the mutual plans developed with the patient before leaving the room and asking if they have any questions about what was discussed is helpful.

Self-reflection or mindfulness is crucial in developing clinical empathy. *Mindfulness* involves being purposefully and nonjudgmentally attentive to your own experience, thoughts, and feelings. ⁴³ Given patients' diverse backgrounds, being consistently respectful and open to individual differences is an ongoing challenge in clinical care. Since clinicians bring their values, assumptions, and biases to every encounter, looking inward is necessary to understand how our expectations and reactions impact our behavior and communication. *Self-reflection* is a continuous part of professional development in clinical work that brings personal awareness to patient care and is one of the most rewarding aspects of it. ^{44, 45}

Box 1-13. Teach-Back Method

- **Plan your approach.** Think about how you will ask your patient to teach back the information. An example would be: “ *We covered a lot today and I want to make sure that I explained things clearly. So, let’s review what we discussed. Can you please describe the three things you agreed to do to help you control your diabetes?*”
- **“Chunk and Check.”** Don’t wait until the end of the visit to initiate teach-back. Chunk out information into small segments and have your patient teach it back. Repeat several times during a visit.
- **Clarify and check again.** If teach-back uncovers a misunderstanding, explain things again using a different approach. Ask patients to teach-back again until they are able to correctly describe the information in their own words. If they parrot your words back to you, they may not have understood.
- **Start slowly and use consistently.** At first, you may want to try teach-back with the last patient of the day. Once you are comfortable with the technique, use teach-back with everyone, every time!
- **Practice.** It will take a little time, but once it is part of your routine, teach-back can be done without awkwardness and does not lengthen a visit.
- **Use the *show-me* method.** When prescribing new medicines or changing a dose, research shows that even when patients correctly say when and how much medicine they will take, many will make mistakes when asked to demonstrate the dose.
- **Use handouts along with teach-back.** Write down key information to help patients remember instructions at home. Point out important information by reviewing written materials to reinforce your patients’ understanding. You can allow patients to refer to handouts when using teach-back, but make sure they use their own words and are not reading the material back verbatim.

Source: Use the Teach-Back Method: Tool #5. Content last reviewed September 2020. Agency for Healthcare Research and Quality. Accessed June 16, 2023. <https://www.ahrq.gov/health-literacy/improve/precautions/tool5.html>

DISPARITIES IN HEALTH CARE

Disparities in risks of disease, morbidity, and mortality are marked and broadly documented across different population groups, reflecting inequities in health care access, income level, type of insurance, educational level, language proficiency, and provider decision making.^{45, 46} This section focuses on important factors that potentiate unequal treatment in the clinical encounter and approaches to help mitigate them.

Social Determinants of Health

Health is significantly affected by the social environment, also known as the *social determinants of health (SDOH)*. The World Health Organization (WHO) defines SDOH as “the conditions in which people are born, grow, work, live, and age, and the wider set of forces and systems shaping the conditions of daily life” (Fig. 1-7). This includes economic policies and systems, development agendas, social norms, social policies, and political systems.⁴⁸ In short, these are the social, economic, and political factors that impact the health of individuals and populations (Box 1-14).⁴⁷ More than their genetic susceptibilities, patients’ health is often influenced by SDOH such as stress,

early life, social exclusion, working conditions, unemployment, social support, addiction, healthy food, and transportation policies. Evidence supports addressing social determinants of health to improve patient health and reduce inequities (Box 1-15).⁴⁹



FIGURE 1-7 . Social determinants of health. (Adapted from HealthyPeople2030 at <https://www.healthypeople.gov>.)

Racism and Bias

Research indicates that implicit clinician biases can negatively impact patient encounters and contribute to health care disparities among different demographic groups.⁵¹ Unconscious mental processes help sort and organize patterns to improve cognitive efficiency, leading to implicit biases formed by society's exposure to imagery, values, media, and emotions depicting stereotypes associated with different demographic groups. Addressing implicit bias in clinical encounters requires understanding its cognitive origins and acknowledging its relation to explicit bias (Box 1-16).⁵³

Box 1-14. Key Social Health Determinant Domains

- **Economic stability** (employment, food insecurity, housing instability, poverty)
- **Education access and quality** (early childhood education and development, enrollment in higher education, high school graduation, language, and literacy)
- **Social and community context** (civic participation, discrimination, incarceration, social cohesion)
- **Health care access and quality** (access to health care, access to primary care, health literacy)
- **Neighborhood and built environment** (access to foods that support healthy eating patterns, crime and violence, environmental conditions, quality of housing)

Source: Healthy People 2030, U.S. Department of Health and Human Services, Office of Disease Prevention and Health Promotion. Accessed June 16, 2023. <https://health.gov/healthypeople/objectives-and-data/social-determinants-health>

Several skills are available that clinicians can use to mitigate the impact of bias in their clinical encounters (Box 1-17).

Box 1-15. Addressing Social Determinants of Health

Level of Interventions	Sample Actions
Patient level	Identify social challenges Offer culturally safe services Help patients access benefits and support services
Practice level	Use patient navigators to ensure care is accessible to those most in need
Community level	Partner with local organizations Engage in health planning Improve environments for health

Box 1-16. Types of Bias

Type of Bias	Description	Example Scenarios
Implicit Bias	Unconscious evaluation of a person based on perceived group identity, leading to negative associations ⁵⁰ Can negatively impact patient encounters and contribute to health care disparities among different demographic groups Can manifest in nonverbal behaviors and contribute to institutional bias	Assuming a female doctor holds a different role, such as a nurse Audibly sighing in frustration about a patient's substance use disorder Poor eye contact and speech errors that may unintentionally convey distrust or discomfort
Explicit Bias	Conscious decision based on beliefs or associations regarding group identity Patient characteristics such as race, gender, sexual orientation, and age can influence communication, diagnostic decision making, treatment recommendations, and nonverbal behaviors	Assuming a particular health condition is more likely to affect patients based on their race. Offering different treatment options based on gender Making negative comments about a patient's age or sexual orientation
Institutional Bias	Refers to the aggregation of implicit biases that lead to a structural system of privilege ^{52 – 56} Misallocates care for marginalized groups	Limiting access to care for certain demographic groups Offering unequal treatment options based on social identities Disregarding patient symptoms or concerns based on assumptions linked to identity

Box 1-17. Skills and Practices to Mitigate Bias in Your Clinical Encounters ^{55 , 56 , 102}

Reflect on patterns of emotion and behavior.	Pay attention to how you feel and how you behave around patients of different identities. The patterns you recognize may reflect biases that impact your interactions with patients as well as your clinical reasoning. Being aware of these biases is the first step in reducing their impact on patient care.
Pause before starting an encounter and prepare for potential triggers of bias	Once you are aware of your potential biases, pay attention to situations that may trigger them. Simply being aware of a bias can help minimize its effect. You may take deliberate actions to reduce the impact of your biases.
Generate alternative hypotheses for biases anchored in behavior	Many biases are anchored in clinician assumptions about observed patient behavior (nonadherence, substance use, etc.). Make it a habit to consider what structural forces (socioeconomic status, race/racism, homophobia, etc.) impact patient behaviors, and how they can challenge assumptions you make about patients.
Practice universal communication and interpersonal skills	Often, clinicians will not recognize when a bias is at play in a clinical encounter. The foundational communication and interpersonal skills described in this book (see Chapter 2 , Interviewing, Communication, and Interpersonal Skills, p. 21) can reduce the impact of such truly unconscious biases on the way you interact with patients.
Explore your patients' identities	Many biases are anchored in clinician assumptions about patient identities. By simply asking patients to clarify what their identities mean to them, clinicians can dismantle their assumptions and better understand their patients. Many approaches to exploring patient identities are presented in this book (see pp. 52–53).
Explore your patients' experiences of bias	Clinical encounters are influenced by patients' prior experiences of implicit and explicit bias in their health care. Exploring

and understanding these experiences can help you be a better partner with your patients. “Unfortunately, many of my patients have had negative experiences with health care. What have been your experiences with health care?”

Source: Burgess DJ, Fu SS, van Ryn M. Why do providers contribute to disparities and what can be done about it? *J Gen Intern Med* . 2004;19(11):1154–1159; Stone J, Moskowitz GB. Non-conscious bias in medical decision making: what can be done to reduce it? *Med Educ* . 2011;45(8):768–776; van Ryn M, Burgess DJ, Dovidio JF, et al. The impact of racism on clinician cognition, behavior, and clinical decision making. *Du Bois Rev* . 2011;8(1):199–218.

Cultural Humility

Cultural humility helps clinicians acknowledge and respect patients’ individuality, mitigate implicit bias, and promote empathy. It involves self-reflection and critique as lifelong learners, examining cultural beliefs and systems to locate points of dissonance or synergy that contribute to patients’ health outcomes.^{57, 58} To moderate disparities in health care, clinicians should engage in self-reflection, critical thinking, and cultural humility as they experience diversity in their clinical practices.^{59–61} This process calls for clinicians to address power imbalances in communication with patients and maintain mutually respectful partnerships with patients and communities (Box 1-18).^{62–66}

Box 1-18. Three Dimensions of Cultural Humility

Self-Awareness

Cultural humility begins with examining our own cultural identity, values, and biases. Overcoming biases, which are natural, but can influence our perceptions of others, is crucial.

For example, a clinician who grew up in a culture that stigmatizes mental health issues may need to challenge their own biases when working with a patient who is seeking treatment for depression. The clinician should recognize their own biases and strive to understand and respect the patient's cultural beliefs and values around mental health.

Respectful Communication

To provide culturally sensitive care, you should let patients lead the conversation about their unique cultural perspectives. Specific questions and an open, respectful attitude can elicit important information. You should be mindful of assumptions and acknowledge your own biases. Learning about a patient's culture can broaden understanding of their needs, but you should avoid stereotyping and research the life experiences of individuals in ethnic or racial groups in their area. Talking to different healers can also expand your knowledge.

For example, a clinician treating a patient from a culture that values natural remedies may need to ask specific questions about the remedies the patient is using and how they fit into the patient's overall treatment plan. The clinician should avoid making assumptions about the effectiveness or safety of these remedies and instead seek to understand the patient's cultural beliefs and values regarding health care.

Collaborative Partnerships

You can promote better health outcomes by cultivating self-awareness and empathy, building collaborative relationships with patients through respectful communication and openness to diverse perspectives. You should validate patients' emotions, remain flexible in treatment planning, and re-examine your own assumptions about clinical care. Success of care is determined by the patient's engagement and investment in your treatment plan.

For example, a clinician may need to collaborate with a patient from a culture that values community involvement in health care decisions. The clinician should be willing to engage with the patient's community and involve them in the patient's treatment plan. The clinician should also be willing to re-examine their own assumptions about what constitutes effective health care and work with the patient to develop a plan that reflects the patient's beliefs and values. The success of the plan will depend on whether the patient feels engaged and invested in the treatment.

Box 1-19. 5Rs of Cultural Humility

	Aim	Ask
Reflection	Clinicians will approach every encounter with humility and understanding that there is always something to learn from everyone.	What did I learn from each person in that encounter?
Respect	Clinicians will treat every person with the utmost respect and strive to preserve dignity at all times.	Did I treat everyone involved in that encounter respectfully?
Regard	Clinicians will hold every person in their highest regard, be aware of, and not allow unconscious biases to interfere in any interactions.	Did unconscious biases drive this interaction?
Relevance	Clinicians will expect cultural humility to be relevant and apply this practice to every encounter.	How was cultural humility relevant in this encounter?
Resiliency	Clinicians will embody the practice of cultural humility to enhance personal resiliency and global compassion.	How was my personal resiliency affected by this interaction?

Source: The 5Rs of Cultural Humility. Reprinted with permission from Society of Hospital Medicine. Accessed June 16, 2023. <https://www.hospitalmedicine.org/practice-management/the-5-rs-of-cultural-humility/>

The 5Rs of Cultural Humility (*reflection, respect, regard, relevance, and resiliency*) is a coaching tool (Box 1-19) that provides clinicians with a concise framework with specific aims and asks to incorporate skills identified at reducing implicit biases in health care. ⁶⁷

OTHER MAJOR CONSIDERATIONS

Other Major Considerations

- Spirituality
- Medical Ethics
- Decisional Capacity
- Approach to a Clinical Ethical Dilemma

Spirituality

Spirituality and religion are sometimes used interchangeably, but it is important to differentiate the two. *Spirituality* is a broader concept that encompasses religion, focusing on universal themes such as meaning, purpose, transcendence, and connection with others. It refers to the way individuals seek and express meaning and purpose and experience their connectedness to the moment, self, others, nature, and the significant and sacred.⁶⁸ *Religion*, on the other hand, involves specific beliefs, practices, texts, and rituals common to a community in relation to something larger than themselves, such as God, the holy, the transcendent, or a higher power.

Respecting patients' religious and spiritual beliefs is essential in culturally competent care. Box 1-20 provides questions to assess spirituality's role in health decisions and coping, while Box 1-21 outlines respectful practices for discussing beliefs in clinical settings.

Medical Ethics

While many clinicians have a general sense of right and wrong, the complexity and uncertainty of clinical situations require a more comprehensive set of ethical principles to guide decision making (Box 1-22).

Medical ethics is a subdiscipline of applied ethics, which itself is a subdiscipline of philosophy. It has an ancient heritage, typically traced back to the Hippocratic Oath, which includes principles such as beneficence, confidentiality, and nonmaleficence. However, this oath also reflected a paternalistic approach to medicine, which remained dominant as the profession became more formalized in the 18th century.^{76, 77}

In the 20th century, there were several events that challenged this dominant ethic of paternalism. Court decisions such as *Schloendorff versus Society of New York Hospital* established the need for clinicians to obtain informed consent from patients before treatment.⁷⁸ Additionally, revelations of physician misconduct, such as the Nazi doctors and the U.S. Tuskegee Syphilis study, led to a further reassessment of medical ethics.^{79, 80}

Box 1-20. Guiding Questions in Assessing the Role of Spirituality in Your Patient

- What values guide your patient's health care decisions?
 - Does your patient consult a religious/spiritual leader before making important health care decisions?
 - Does your patient have particular religious/spiritual concerns about blood products or porcine implants?
- How do patient spiritual beliefs and practices influence how they cope with their illness and care for themselves? ¹⁰³
 - Is there a significant community involved who may aid them while they are sick?
 - Is there a spiritual practice such as yoga or meditation that may aid them in their healing?
- Does your patient have spiritual struggle or distress and need a referral to a chaplain?
 - *Spiritual struggle* is defined as "... tensions, conflicts, and questions over sacred matters within oneself, with others, and with God." ¹⁰⁴ Examples include feeling abandoned by God or one's religious community, a belief that the universe is out to get you, or doubts about one's fundamental beliefs and values.
 - Spiritual struggle has been shown to increase depressive symptoms, emotional distress, and risk of mortality while decreasing physical health, quality of life, and recovery of independence in daily activities. ¹⁰⁵

Box 1-21. Do's and Don'ts for Religious and Spiritual Beliefs in Clinical Encounters

Don't assume that patients are religious.	In the United States, 27% of adults identify as "spiritual, but not religious." ^{70, 71}
Don't assume that patients are not religious.	Nearly 75% of U.S. adults identify as religious. ⁷²
Don't assume that you know what religion or spirituality means to the patient.	Patients often customize their beliefs and practices, so taking a spiritual history is important.
Don't assume that religion or spirituality has no effect on health.	Religion and spirituality can be social determinants of health and impact health outcomes positively or negatively. For example, Seventh-Day Adventists who follow a vegetarian diet for religious reasons live an average of 10 years longer than most Americans. On the other hand, medically ill older adult patients who feel that God has abandoned them have an increased risk of death. ^{73 – 75}
Do take a spiritual history.	Ask patients about their spiritual or religious beliefs, practices, and community involvement to understand how they may affect their health and health care decisions.
Do use nonjudgmental language.	Avoid imposing your beliefs on the patient or making assumptions. Use open-ended questions to encourage patients to share their beliefs and experiences.
Do be respectful of the patient's beliefs.	Show empathy and understanding toward their religious or spiritual practices and support their autonomy in making health care decisions that align with their beliefs.
Do be aware of resources available.	Accessing resources like chaplains or spiritual advisors can help provide additional support to patients.

Box 1-22. Core Values of Medical Ethics

- **Nonmaleficence** (“*first, do no harm*”) directive that health care professionals should avoid causing harm to patients and minimize the negative effects of treatments.
- **Beneficence** dictum that clinicians are to act for the patients’ good by preventing or treating disease.
- **Respect for autonomy** commitment to accept the choices patients with decisional capacity make about which treatments to undergo, including to reject treatment. The addition of this value to medical ethics changed the clinician–patient relationship from a paternalistic one to a more collaborative one.
- **Decisional capacity** ability to make an autonomous choice that clinicians should respect.⁸³
- **Confidentiality** duty to prevent the disclosure of patients’ personal information to parties who are not authorized to learn that information.
- **Informed consent** principle that clinicians must elicit patients’ voluntary and informed authorization to test or treat them for illness or injury. Because patients cannot consent to treatment without knowing what they are being treated for, this principle also encompasses the responsibility to inform patients of diagnoses, prognoses, and treatment alternatives.
- **Truth telling** value that clinicians should disclose information beyond that required by informed consent that may be relevant to patients (e.g., the number of similar procedures a physician has performed).
- **Justice** value that all patients with similar medical needs should receive similar medical treatment and should be treated fairly by clinicians.

As a response, there was a “bioethics revolution” in the mid-20th century, led by philosophers and theologians, which retained the older principles of Hippocratic and professional values, while also establishing respect for patient autonomy as a core value of medicine. This respect for autonomy empowered patients to make health choices that reflect their own views of what is good for them. Respect for autonomy, along with beneficence, nonmaleficence, and justice, became established as the common core of health care ethics and are now incorporated into most professional codes for health care providers.^{81, 82}

Decisional Capacity

Capacity and competence are two distinct concepts in health care. *Capacity* is a clinical designation that clinicians can assess, while *competence* is a judicial determination that only a court can make. Determining a patient’s capacity to make informed health care decisions is important, and it involves assessing their ability to communicate a choice, understand relevant information, appreciate the situation and its consequences, and reason about treatment options ([Box 1-23](#)).⁸³

Box 1-23. Elements of Decisional Capacity

Patients must have the ability to:

- Understand the relevant information about proposed diagnostic tests or treatment
- Appreciate their situation (including their underlying values and current clinical situation)
- Use reason to make a decision
- Communicate their choice

Source: Sessums LL, Zembruska H, Jackson JL. Does this patient have medical decision-making capacity? *JAMA* . 2011;306(4):420–427.

If a patient lacks capacity, identifying a health care proxy or surrogate decision maker is necessary, and this role may be assumed by a spouse or family member if the patient has not designated one. Importantly, decisional capacity is both temporal and situational and can fluctuate depending on the patient's condition and the complexity of the decision at hand.^{84, 85}

Approach to a Clinical Ethical Dilemma

Medical ethics is a crucial aspect of every clinical encounter with patients, even though it may not always be explicitly considered. As students, you will be exposed to ethical challenges that you may encounter in your future practice as clinicians. While adhering to ethical values becomes a natural part of clinical practice through training, some complex cases may require explicit and critical reflection to determine the ethical course of action. In such situations, heuristics can provide guidance on how to approach ethical dilemmas (Box 1-24).^{86–88} While these practical methods may not guarantee an optimal or perfect solution, they can help achieve an immediate goal and lead to a satisfactory solution in situations where finding an optimal one is impossible or impractical.

Box 1-24. How to Resolve a Clinical Ethical Dilemma

Step	Description
1	Formulate an ethics question that summarizes the challenge they face in an ethically complex clinical situation.
2	Collect all of the information that clinicians believe is relevant to the case, including clinical information, patient's goals, preferences, cultural or religious commitments, financial resources, and interests and concerns of other stakeholders.
3	Identify the ethical principles and guidelines, including laws, institutional policies, and ethics concepts, that would guide the clinician's conduct in the case.
4	Reflect on how the relevant principles identified in the prior step would guide their conduct in the case and clarify the ethical dilemma if there is any.
5	Evaluate the different options and decide which ethical concept is the most important in the case and follow its guidance.
6	Make an action plan on how to communicate the decision and plan how to implement the ethical course of action. Enlist institutional support from an ethics consultant or committee and other services, such as social work services, if necessary.

REFERENCES

- Athreya BH. *Handbook of Clinical Skills: A Practical Manual*. World Scientific Publishing Co.; 2010.
- AAMC Task Force On the Clinical Skills Education of Medical Students. *Recommendations for Clinical Skills Curricula for Undergraduate Medical Education*. Association of American Medical Colleges; 2005. Accessed June 16, 2023. https://store.aamc.org/downloadable/download/sample/sample_id/174
- Fortin AH VI, Dwamena FC, Frankel RM, Smith RC. *Smith's Patient-Centered Interviewing: An Evidence-Based Method*. 3rd ed. The McGraw-Hill Companies, Inc; 2012.
- Smith RC. An evidence-based infrastructure for patient-centered interviewing. In: Frankel RM, Quill TE, McDaniel SH, eds. *The Biopsychosocial Approach: Past, Present, and Future*. University of Rochester Press; 2003:148.
- Kleinman A, Eisenberg L, Good B. Culture, illness, and care: clinical lessons from anthropologic and cross-cultural research. *Ann Intern Med*. 1978;88(2):251–258.
- Mauksch L, Farber S, Greer HT. Design, dissemination, and evaluation of an advanced communication elective at seven U.S. medical schools. *Acad Med*. 2013;88(6):843–851.
- Haidet P, Paterniti DA. “Building” a history rather than “taking” one: a perspective on information sharing during the medical interview. *Arch Intern Med*. 2003;163(10):1134–1140.
- Stewart M, Brown JB, Weston WW, McWhinney IR, McWilliam CL, Freeman TR. *Patient-Centered Medicine: Transforming the Clinical Method*. 2nd ed. Radcliffe Medical Press Ltd; 2003.
- Atlas SJ, Grant RW, Ferris TG, Chang Y, Barry MJ. Patient-physician connectedness and quality of primary care. *Ann Intern Med*. 2009;150(5):325–335.
- van de Poel K, Vanagt E, Schimpf U, Gasiorek J. *Communication Skills for Foreign and Mobile Medical Professionals*. Springer; 2013.
- Kurtz S, Silverman J, Benson J, Draper J. Marrying content and process in clinical method teaching: enhancing the Calgary–Cambridge guides. *Acad Med*. 2003;78(8):802–809.
- Kurtz S, Silverman J, Draper J. *Teaching and Learning Communication Skills in Medicine*. 2nd ed. Radcliffe Medical Press; 1998.
- Kurtz SM, Silverman JD. The Calgary–Cambridge Referenced Observation Guides: an aid to defining the curriculum and organizing the teaching in communication training programmes. *Med Educ*. 1996;30(2):83–89.
- de Haes H, Bensing J. Endpoints in medical communication research, proposing a framework of functions and outcomes. *Patient Educ Couns*. 2009;74(3):287–294.
- Tomsik PE, Witt AM, Raddock ML, et al. How well do physician and patient visit priorities align? *J Fam Pract*. 2014;63(8):E8–E13.
- Suchman AL, Matthews DA. What makes the patient-doctor relationship therapeutic? Exploring the connexional dimension of medical care. *Ann Intern Med*. 1988;108(1):125–130.
- Matthews DA, Suchman AL, Branch WT Jr. Making “connexions”: enhancing the therapeutic potential of patient-clinician relationships. *Ann Intern Med*. 1993;118(12):973–977.
- Larson EB, Yao X. Clinical empathy as emotional labor in the patient-physician relationship. *JAMA*. 2005;293(9):1100–1106.
- Krasner MS, Epstein RM, Beckman H, et al. Association of an educational program in mindful communication with burnout, empathy, and attitudes among primary care physicians. *JAMA*. 2009;302(12):1284–1293.
- Deutsch MB, Buchholz D. Electronic health records and transgender patients—practical recommendations for the collection of gender identity data. *J Gen Intern Med*. 2015;30(6):843–847.
- Makoul G, Zick A, Green M. An evidence-based perspective on greetings in medical encounters. *Arch Intern Med*. 2007;167(11):1172–1176.
- Meiri N, Ankri A, Hamad-Saied M, Konopnicki M, Pillar G. The effect of medical clowning on reducing pain, crying, and anxiety in children aged 2–10 years old undergoing venous blood drawing—a randomized controlled study. *Eur J Pediatr*. 2016;175(3):373–379.
- Meiri N, Ankri A, Zidan F, et al. Assistance of medical clowns improves the physical examinations of children aged 2–6 years. *Isr Med Assoc J*. 2017;19(12):786–791.
- Damm L, Leiss U, Habeler U, Ehrich J. Improving care through better communication: understanding the benefits. *J Pediatr*. 2015;166(5):1327–1328.
- Drutz JE, White-Satcher D. The pediatric physical examination: general principles and standard measurements. *UpToDate*. 2023. Accessed November 5, 2024. www.uptodate.com/contents/the-pediatric-physical-examination-general-principles-and-standard-measurements
- Berlan ED, Bravender T. Confidentiality, consent, and caring for the adolescent patient. *Curr Opin Pediatr*. 2009;21(4):450–456.
- Gilbert AL, Rickert VI, Aalsma MC. Clinical conversations about health: the impact of confidentiality in preventive adolescent care. *J Adolesc Health*. 2014;55(5):672–677.
- Lewis Gilbert A, McCord AL, Ouyang F, et al. Characteristics associated with confidential consultation for adolescents in primary care. *J Pediatr*. 2018;199:79–84.e1.
- Friedman MR, Dodge B, Schick V, et al. From bias to bisexual health disparities: attitudes toward bisexual men and women in the United States. *LGBT Health*. 2014;1(4):309–318.
- Polek CA, Hardie TL, Crowley EM. Lesbians’ disclosure of sexual orientation and satisfaction with care. *J Transcult Nurs*. 2008;19(3):243–249.

31. Durso LE, Meyer IH. Patterns and predictors of disclosure of sexual orientation to healthcare providers among lesbians, gay men, and bisexuals. *Sex Res Social Policy* . 2013;10(1):35–42.
32. Strutz KL, Herring AH, Halpern CT. Health disparities among young adult sexual minorities in the U.S. *Am J Prev Med* . 2015;48(1):76–88.
33. Beckman HB, Frankel RM. The effect of physician behavior on the collection of data. *Ann Intern Med* . 1984;101(5):692–696.
34. Jackson JL, Passamonti M, Kroenke K. Outcome and impact of mental disorders in primary care at 5 years. *Psychosom Med* . 2007;69(3):270–276.
35. Lang F, Floyd MR, Beine KL. Clues to patients' explanations and concerns about their illnesses. A call for active listening. *Arch Fam Med* . 2000;9(3):222–227.
36. Behforouz HL, Drain PK, Rhatigan JJ. Rethinking the social history. *N Engl J Med* . 2014;371(14):1277–1279.
37. Robbins JA, Bertakis KD, Helms LJ, Azari R, Callahan EJ, Creten DA. The influence of physician practice behaviors on patient satisfaction. *Fam Med* . 1993;25(1):17–20.
38. Agency for Healthcare Research and Quality. Use the Teach-Back Method: Tool #5. Content last reviewed September 2020. Accessed June 16, 2023. <https://www.ahrq.gov/health-literacy/improve/precautions/tool5.html>
39. Kripalani S, Jackson AT, Schnipper JL, Coleman EA. Promoting effective transitions of care at hospital discharge: a review of key issues for hospitalists. *J Hosp Med* . 2007;2(5):314–323.
40. Kemp EC, Floyd MR, McCord-Duncan E, Lang F. Patients prefer the method of “tell back-collaborative inquiry” to assess understanding of medical information. *J Am Board Fam Med* . 2008;21(1):24–30.
41. Barry MJ, Edgman-Levitan S. Shared decision making—pinnacle of patient-centered care. *N Engl J Med* . 2012;366(9):780–781.
42. Elwyn G, Frosch D, Thomson R, et al. Shared decision making: a model for clinical practice. *J Gen Intern Med* . 2012;27(10):1361–1367.
43. Epstein RM. Mindful practice. *JAMA* . 1999;282(9):833–839.
44. Beach MC, Roter D, Korthuis PT, et al. A multicenter study of physician mindfulness and health care quality. *Ann Fam Med* . 2013;11(5):421–428.
45. Institute of Medicine (US) Committee on Understanding and Eliminating Racial and Ethnic Disparities in Health Care. Smedley BD, Stith AY, Nelson AR, eds. *Unequal Treatment: Confronting Racial and Ethnic Disparities in Health Care* . National Academies Press (US); 2003.
46. *National Healthcare Disparities Report, 2013* . Agency for Healthcare Research and Quality; 2014. <https://archive.ahrq.gov/research/findings/nhqrdr/nhdr13/index.html>
47. Lucyk K, McLaren L. Taking stock of the social determinants of health: a scoping review. *PLoS One* . 2017;12(5):e0177306.
48. Wilkinson R, Marmot M, eds. *Social Determinants of Health: The Solid Facts* . 2nd ed., World Health Organization; 2003. Accessed November 5, 2024. <https://iris.who.int/handle/10665/326568>
49. Andermann A; CLEAR Collaboration. Taking action on the social determinants of health in clinical practice: a framework for health professionals. *CMAJ* . 2016;188(17–18):E474–E483.
50. FitzGerald C, Hurst S. Implicit bias in healthcare professionals: a systematic review. *BMC Med Ethics* . 2017;18(1):19.
51. *Racial Disparities in Health Care: Confronting Unequal Treatment, Hearing Before the Subcommittee on Criminal Justice, Drug Policy and Human Resources of the Committee on Government Reform* . Second Session. U.S. Government Printing Office; 2002. <https://www.govinfo.gov/content/pkg/CHRG-107hhr86436/pdf/CHRG-107hhr86436.pdf>
52. Chapman EN, Kaatz A, Carnes M. Physicians and implicit bias: how doctors may unwittingly perpetuate health care disparities. *J Gen Intern Med* . 2013;28(11):1504–1510.
53. Gordon HS, Street RL Jr, Sharf BF, Soucek J. Racial differences in doctors' information-giving and patients' participation. *Cancer* . 2006;107(6):1313–1320.
54. Penner LA, Blair IV, Albrecht TL, Dovidio JF. Reducing racial health care disparities: a social psychological analysis. *Policy Insights Behav Brain Sci* . 2014;1(1):204–212.
55. Burgess DJ, Fu SS, van Ryn M. Why do providers contribute to disparities and what can be done about it? *J Gen Intern Med* . 2004;19(11):1154–1159.
56. Stone J, Moskowitz GB. Non-conscious bias in medical decision making: what can be done to reduce it? *Med Educ* . 2011;45(8):768–776.
57. Tervalon M, Murray-García J. Cultural humility versus cultural competence: a critical distinction in defining physician training outcomes in multicultural education. *J Health Care Poor Underserved* . 1998;9(2):117–125.
58. Tervalon M. Components of culture in health for medical students' education. *Acad Med* . 2003;78(6):570–576.
59. Like RC. Educating clinicians about cultural competence and disparities in health and health care. *J Contin Educ Health Prof* . 2011;31(3):196–206.
60. Boutin-Foster C, Foster JC, Konopasek L. Viewpoint: physician, know thyself: the professional culture of medicine as a framework for teaching cultural competence. *Acad Med* . 2008;83(1):106–111.
61. Teal CR, Street RL. Critical elements of culturally competent communication in the medical encounter: a review and model. *Soc Sci Med* . 2009;68(3):533–543.
62. Smith WR, Betancourt JR, Wynia MK, et al. Recommendations for teaching about racial and ethnic disparities in health and health care. *Ann Intern Med* . 2007;147(9):654–665.
63. Embedding cultural diversity and cultural and linguistic competence: a guide for UCEDD curricula and training activities. Georgetown University Center for Child and Human Development; District of Columbia's University Center for Excellence in Developmental Disabilities; 2017. Accessed June 16, 2023. <https://gucchd.georgetown.edu/resources.php>
64. Juarez JA, Marvel K, Brezinski KL, Glazner C, Towbin MM, Lawton S. Bridging the gap: a curriculum to teach residents cultural humility. *Fam Med* . 2006;38(2):97–102.

65. Labib MA, Abou-Al-Shaar H, Cavallo C. Minimally invasive cranial neurosurgery in the 21st century. *J Neurosurg Sci* . 2018;62(6):615–616.
66. Jacobs EA, Rolle I, Ferrans CE, Whitaker EE, Warnecke RB. Understanding African Americans' views of the trustworthiness of physicians. *J Gen Intern Med* . 2006;21(6):642–647.
67. Masters C, Robinson D, Faulkner S, Patterson E, McIlraith T, Ansari A. Addressing biases in patient care with the 5Rs of cultural humility, a clinician coaching tool. *J Gen Intern Med* . 2019;34(4):627–630.
68. Puchalski C, Ferrell B, Virani R, et al. Improving the quality of spiritual care as a dimension of palliative care: the report of the Consensus Conference. *J Palliat Med* . 2009;12(10):885–904.
69. Whitley R. Religious competence as cultural competence. *Transcult Psychiatry* . 2012;49(2):245–260.
70. Lipka M, Gecewicz C. More Americans now say they're spiritual but not religious. Pew Research Center; 2017. <https://www.pewresearch.org/fact-tank/2017/09/06/more-americans-now-say-theyre-spiritual-but-not-religious/>
71. Oppenheimer M. When some turn to church, others go to CrossFit. *New York Times* . November 27, 2015. <https://www.nytimes.com/2015/11/28/us/some-turn-to-church-others-to-crossfit.html>
72. Pew Research Center. Religious landscape study. <https://www.pewforum.org/religious-landscape-study>
73. Idler EL, ed. *Religion as a Social Determinant of Public Health* . Oxford University Press; 2014.
74. Fraser GE, Shavlik DJ. Ten years of life: is it a matter of choice? *Arch Intern Med* . 2001;161(13):1645–1652.
75. Pargament KI, Koenig HG, Tarakeshwar N, Hahn J. Religious struggle as a predictor of mortality among medically ill elderly patients: a 2-year longitudinal study. *Arch Intern Med* . 2001;161(15):1881–1885.
76. Baker RB, McCullough LB. What is the history of medical ethics? In: Baker RB, McCullough LB, eds. *The Cambridge World History of Medical Ethics* . Cambridge University Press; 2009:3–15.
77. McCullough LB. Contributions of ethical theory to pediatric ethics: pediatricians and parents as co-fiduciaries of pediatric patients. In: Miller G, ed. *Pediatric Bioethics* . Cambridge University Press; 2010:11–21.
78. *Schloendorff v. Society of the New York Hospital* , 105 NE 92 (211 NY 125 1914).
79. White BD, Shelton WN, Riva CJ. Were the “pioneer” clinical ethics consultants “outsiders”? For them, was “critical distance” that critical? *Am J Bioeth* . 2018;18(6):34–44.
80. Fox RC, Swazey JP. *Observing Bioethics* . Oxford University Press; 2008.
81. Baker R. *Before Bioethics: A History of American Medical Ethics from the Colonial Period to the Bioethics Revolution* . Oxford University Press; 2013.
82. Jonsen AR. *The Birth of Bioethics* . Oxford University Press; 1998.
83. Appelbaum PS. Clinical practice. Assessment of patients' competence to consent to treatment. *N Engl J Med* . 2007;357(18):1834–1840.
84. Sessums LL, Zembrzuska H, Jackson JL. Does this patient have medical decision-making capacity? *JAMA* . 2011;306(4):420–427.
85. Joint Centre for Bioethics. Aid to Capacity Evaluation (ACE). University of Toronto, 2003. Accessed June 16, 2023. <https://jcb.utoronto.ca/wp-content/uploads/2021/03/ace.pdf>
86. *Core Competencies for Healthcare Ethics Consultation* . American Society for Bioethics and Humanities; 2011.
87. Shamoo AE, Resnik DB. *Responsible Conduct of Research* . 2nd ed. Oxford University Press; 2009.
88. DeLamater JD, Myers DJ. *Social Psychology* . 7th ed. Cengage Learning; 2010.
89. World Health Organization. Disability and Health. *World Health Organization* , 2023. Accessed November 5, 2024. www.who.int/news-room/fact-sheets/detail/disability-and-health
90. Institute on Disability, University of New Hampshire. 2023 *Disability Statistics Annual Report* . ERIC. Accessed November 5, 2024. <https://eric.ed.gov/?id=ED628657>
91. Ward BW, Dahlhamer JM, Galinsky AM, Joestl SS. Sexual orientation and health among U.S. adults: national health interview survey, 2013. *Natl Health Stat Report* . 2014;(77):1–10.
92. Gates GJ. Demographics and LGBT health. *J Health Soc Behav* . 2013;54(1):72–74.
93. Ahmad F, Hogg-Johnson S, Stewart DE, Skinner HA, Glazier RH, Levinson W. Computer-assisted screening for intimate partner violence and control: a randomized trial. *Ann Intern Med* . 2009;151(2):93–102.
94. United States Census Bureau. Same-Sex Couple Households: Characteristics by Sex of Spouses or Partners. *United States Census Bureau* . Accessed November 5, 2024. <https://www.census.gov/data/tables/time-series/demo/same-sex-couples/ssc-house-characteristics.html>
95. Institute of Medicine (US) Committee on Lesbian, Gay, Bisexual, and Transgender Health Issues and Research Gaps and Opportunities. *The Health of Lesbian, Gay, Bisexual, and Transgender People: Building a Foundation for Better Understanding* . National Academies Press (US); 2011. <https://www.ncbi.nlm.nih.gov/books/NBK64806/>
96. Tomczyk S, Bennett NM, Stoecker C, et al; Centers for Disease Control and Prevention (CDC). Use of 13-valent pneumococcal conjugate vaccine and 23-valent pneumococcal polysaccharide vaccine among adults aged ≥65 years: recommendations of the Advisory Committee on Immunization Practices (ACIP). *MMWR Morb Mortal Wkly Rep* . 2014;63(37):822–825.
97. Centers for Disease Control and Prevention. 6440: National Health Interview Survey, 2022. *CDC Blogs* . Accessed November 5, 2024. <https://blogs.cdc.gov/nchs/2022/05/25/6440/>
98. James SE, Herman JL, Rankin S, Keisling M, Mottet L, Anafi M. *The Report of the 2015 U.S. Transgender Survey* . National Center for Transgender Equality; 2016. Accessed June 16, 2023. <https://transequality.org/sites/default/files/docs/usts/USTS-Full-Report-Dec17.pdf>
99. Ventres W, Kooienga S, Vuckovic N, Marlin R, Nygren P, Stewart V. Physicians, patients, and the electronic health record: an ethnographic analysis. *Ann Fam Med* . 2006;4(2):124–131.
100. Communication: what do patients want and need? *J Oncol Pract* . 2008;4(5):249–253.

101. Pollak KI, Arnold RM, Jeffreys AS, et al. Oncologist communication about emotion during visits with patients with advanced cancer. *J Clin Oncol* . 2007;25(36):5748–5752.
102. van Ryn M, Burgess DJ, Dovidio JF, et al. The impact of racism on clinician cognition, behavior, and clinical decision making. *Du Bois Rev* . 2011;8(1):199–218.
103. Puchalski C, Romer AL. Taking a spiritual history allows clinicians to understand patients more fully. *J Palliat Med* . 2000;3(1):129–137.
104. Exline JJ, Rose ED. Religious and spiritual struggles. In: Paloutzian RF, Park CL, eds. *Handbook of the Psychology of Religion and Spirituality* . 1st ed. Guilford Press; 2005:380–398.
105. Fitchett G, Risk JL. Screening for spiritual struggle. *J Pastoral Care Counsel* . 2009;63(1–2):4–12.

CHAPTER 2

Interviewing, Communication, and Interpersonal Skills

Choosing to enter the health care professions is a significant decision, often driven by the desire to foster effective, healing relationships—a cornerstone of patient care. ¹ This chapter delves into the essential techniques of therapeutic interviewing, skills that require continual refinement throughout your career. Such skills are not innate; they necessitate diligent practice and constructive feedback from mentors to enhance your proficiency. As you gain experience, you will become adept at selecting the most appropriate techniques to navigate the complex dynamics of human behavior within patient relationships.

Chapter 1 introduced the concept that clinical interviewing transcends a mere exchange of questions and answers. It demands a nuanced sensitivity to patients' emotional and behavioral signals (Fig. 2-1). This approach allows you to weave together a patient's narrative in a way that is both dynamic and responsive to their cues, emotions, and concerns. ²



FIGURE 2-1 . The interviewing process using effective communication skills.

Chapter 1 also highlighted that the competencies required for effective interviewing diverge significantly from those used in compiling a health history. While the health history format is crucial for categorizing a patient's medical story into relevant sections (present, past, and family health), it serves a different purpose from the interview process. The interview is about building a connection and understanding, whereas the health history provides a structured overview of the patient's medical background. As you explore the art of skilled interviewing in this chapter, appreciate the distinct yet complementary roles of these two aspects.

See Chapter 3 , Health History, for the format of the health history, pp. 47–61.

Chapter Content Guide

- Organizing Patient–Clinician Communication Strategies
- Establishing the Foundation of Communication
- Building the Interaction
- Strengthening the Relationship
- Addressing Sensitive Topics
- Practicing Legal and Ethical Communication
- Using Advanced Communication Strategies
- Navigating Complex Discussions
- Encouraging Patient Self-Efficacy
- Enhancing Interprofessional Communication
- Managing Challenging Patient Situations and Behaviors
- Being Patient-Centered in Computerized Clinical Settings
- Learning Communication Skills from Standardized Patients

ORGANIZING PATIENT–CLINICIAN COMMUNICATION STRATEGIES

Communication skills refer to the ability to convey information effectively and efficiently in various contexts. These skills include listening, speaking, observing, empathizing, and using diplomacy and tact. Communication skills encompass the capacity to engage in meaningful interaction with others. They are fundamental to successful relationships, both personal and professional.

You may recall that the clinical encounter has a structure and sequence: *initiating the session, information gathering, physical examination, explaining and planning, and closing the session*.^{3–5} In this section, we will consider the global communication and interpersonal techniques that can be used across all stages of the clinical encounter.

This chapter is organized to reflect the progression of a typical patient–clinician encounter, highlighting foundational communication skills, building rapport, addressing sensitive topics, and navigating through complex discussions and ethical considerations.

At the outset, **establishing the foundation of communication** focuses on initiating the interaction through active or attentive listening and empathic responses. These initial steps are crucial for building trust and understanding, setting the stage for a more in-depth exploration of the patient's concerns.

As the interaction progresses, the focus shifts to **building the interaction** through guided questioning and specific techniques aimed at eliciting detailed information from the patient. This

segment ensures that the clinician gathers comprehensive data necessary for accurate diagnosis and treatment planning.

The **strengthening the relationship** section explores enhancing the patient–clinician relationship through summarization, transitions, validation, partnering, and empowering the patient. These strategies emphasize the importance of collaboration and shared decision-making in the therapeutic process.

Addressing sensitive topics and ethical considerations acknowledges the challenges of broaching sensitive subjects and the complexities of informed consent and working with medical interpreters. This part underscores the significance of ethical communication in maintaining patient dignity and autonomy.

Finally, **advanced communication strategies** and **navigating complex discussions** cover the delivery of serious news and advance care planning, while **encouraging patient self-efficacy** focuses on motivational interviewing (MI) as a technique to foster behavior change for improved health outcomes.

This structured approach to patient–clinician communication aims to enhance the effectiveness of clinical encounters, ensuring that patients feel heard, understood, and actively involved in their care.

ESTABLISHING THE FOUNDATION OF COMMUNICATION

Active (Attentive) Listening

Active listening or *attentive listening* lies at the heart of the patient interview. It involves several specific skills that enhance, guide, and structure your interaction. It involves intently focusing on the patient’s messages, empathizing with their emotional state, and using both verbal and nonverbal cues to encourage them to share more about their feelings and concerns. Through active listening, you can connect with the patient on various levels of their experience, which requires practice.⁶ It is common to become preoccupied with your next question or potential diagnoses, leading to a loss of focus on the patient’s narrative. Therefore, concentrate on both what the patient is explicitly saying and the unspoken cues their body language may reveal, as these can sometimes convey a message that differs from their words.

BUILDING THE INTERACTION

Guided Questioning

Several strategies can help you gather more information without disrupting the patient’s narrative flow. The objective is to enable comprehensive communication, allowing the patient to express themselves fully in their own words, without interruptions. With guided questions, you demonstrate ongoing interest in the patient’s emotions and revelations (Box 2-1).⁷ This approach prevents questions that might predetermine the direction of the conversation or inadvertently silence the patient. A series of “yes–no” questions can make the patient feel constrained and passive, resulting in

a notable loss of detail. Guided questioning is a more effective way to capture the entirety of the patient's story.

By strategically using a variety of questioning techniques, clinicians can navigate the complexities of patient narratives, uncovering vital information that might otherwise remain obscured.

Box 2-1. Techniques of Guided Questioning

- Moving from open-ended to focused questions
- Using questioning that elicits a graded response
- Asking a series of questions, one at a time
- Offering multiple choices for answers
- Clarifying what the patient means
- Encouraging with continuers
- Using echoing/repetition

Moving from Open-Ended to Focused Questions. After inviting patients to share their story in their own words using open-ended questions, your questions become more focused, homing in on specific aspects of the patient's experience. This gradual narrowing of focus helps clinicians gather comprehensive data, while allowing patients to express themselves fully (Fig. 2-2).

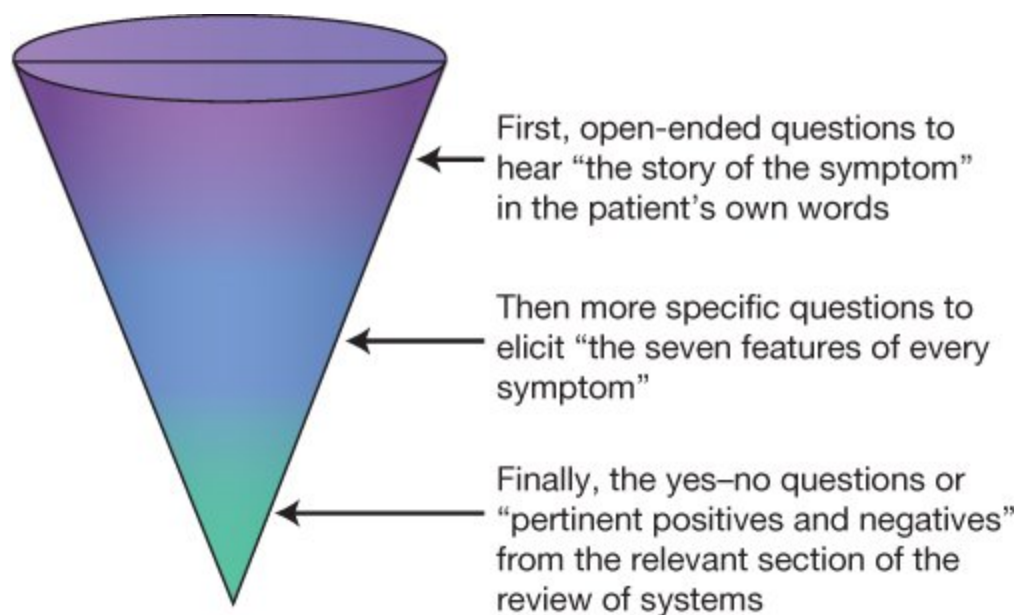


FIGURE 2-2 . Guided questioning from open-ended to more focused questions.

Start with a genuinely open-ended inquiry that does not imply a specific answer. An effective sequence might look like this:

- "Describe your chest discomfort." (Pause)
- "What more can you tell me?" (Pause)
- "Where exactly did you feel it?" (Pause) "Please show me."
- "Did you feel it anywhere else?" (Pause) "Did the sensation move?" (Pause) "Which arm did it move to?"

Steer clear of *leading questions* that suggest a particular answer or response, such as: “*Has your pain been getting better?*” or “*You haven’t noticed any blood in your stools, have you?*” Asking, “*Does your pain feel like pressure?*” may lead to a simple “*Yes*” that cuts short the opportunity for the patient to provide a detailed description of their experience. Instead, use a more neutral prompt like, “*Please describe your pain,*” to encourage a comprehensive response.

Questioning that Elicits a Graded Response. Rather than settling for binary answers, this approach seeks to quantify experiences or symptoms, providing a richer, more nuanced understanding of the patient’s condition.

Ask questions that require a graded response, providing deeper insight into the patient’s condition, rather than a yes–no answer. For example, asking “*How many steps can you climb before feeling short of breath?*” is more informative than the straightforward “*Do you get short of breath climbing stairs?*”

Asking a Series of Questions, One at a Time. This clear, methodical approach prevents overwhelming the patient, ensuring that each question is given the attention it deserves and that answers are as informative as possible.

Be sure to ask one question at a time. Bundling inquiries such as, “*Any history of tuberculosis, diabetes, asthma, heart conditions, or high blood pressure in your family?*” might simply elicit a “*No*” due to confusion. Instead, frame your questions clearly, for example: “*Do you have any of the following health issues?*” Make sure to pause and establish eye contact before listing each condition individually. This approach helps maintain clarity and encourages more accurate responses.

Offering Multiple Choices for Answers. Sometimes, patients need help in describing their symptoms. By providing a range of options, clinicians can help patients better articulate their experiences, leading to more accurate and detailed responses.

To minimize bias, present multiple-choice options when asking questions. For example, inquire about the nature of pain by asking, “*Which of the following words best describes your pain: aching, sharp, pressing, burning, shooting, or something else?*” This method can be applied broadly; nearly every specific question can offer a choice between two alternatives, such as asking, “*Is your cough accompanied by phlegm, or is it dry?*” This approach encourages more precise responses and helps in gathering detailed information without leading the patient.

Clarifying What the Patient Means. This critical step ensures that the clinician’s understanding aligns with the patient’s intent, avoiding misinterpretations that could lead to misguided conclusions.

Phrases like “*Could you explain what you mean by ‘the flu’?*” or “*You mentioned you were acting like your mother. Can you elaborate on that?*” are essential for this purpose. Taking the time to request such details conveys your genuine interest in comprehending the patient’s story, which not only reassures them but also strengthens the therapeutic relationship.

Encouraging with Continuers. Nonverbal cues and verbal affirmations signal that you are engaged and interested, encouraging the patient to delve deeper into their narrative.

Even without speaking, your posture and gestures (*nonverbal encouragements*), as well as simple verbal affirmations (*neutral utterances*), can motivate the patient to share more. A pause, a nod, or maintaining attentive and relaxed silence signals to the patient to proceed. Leaning in, establishing eye contact, and using phrases such as “*Uh-huh,*” “*Go on,*” or “*I’m listening*” all contribute to fostering an environment where the patient feels encouraged to continue their narrative.

Echoing (Repetition). This simple yet powerful technique reflects the patient's own words back to them, prompting further elaboration and demonstrating a genuine interest in their story. For instance, consider the following exchange:

- Patient: *"The pain got worse and started to spread."* (Pause)
- Response: *"Spread?"* (Pause)
- Patient: *"Yes, it moved to my shoulder and traveled down my left arm to my fingers. It was so severe I feared for my life."* (Pause)
- Response: *"Feared for your life?"*
- Patient: *"Yes, it reminded me of the pain my father experienced during his heart attack, which made me worry I was undergoing the same ordeal."*

Using this reflective technique not only uncovers the pain's location and intensity but also its significance to the patient. This approach avoids skewing the narrative or disrupting the patient's thought process.

STRENGTHENING THE RELATIONSHIP

Empathic Responses

Empathic responses play a crucial role in building patient rapport and fostering healing. ^{8, 9} *Empathy* is often described as the ability to put yourself in the patient's shoes, to genuinely feel their pain as if it were your own, and to offer support in a compassionate manner. ¹⁰ It entails a readiness to share in the patient's suffering, an integral aspect of the healing process. ¹¹ As patients communicate with you, they might reveal emotions through their words or facial expressions that they have not fully recognized themselves. Recognizing these unspoken feelings is key to understanding their health issues.

To effectively express empathy, first acknowledge the patient's emotions, then actively seek and engage with the emotional aspects of their experience. ^{12, 13} Initially, delving into these emotions may feel uncomfortable, but such empathic engagement is vital for building deeper trust. When you pick up on subtle cues of unvoiced emotions, whether through facial expressions, tone of voice, behaviors, or their words, gently probe with questions like, *"How do you feel about that?"* or *"It seems like this is troubling you; can you tell me more?"*

Be aware that a patient's feelings may not always align with your initial perceptions. For example, assuming that a parent's death is solely a source of grief might overlook the patient's relief from a burdensome relationship. Instead, asking open-ended questions like, *"You've lost your father. What has that experience been like for you?"* encourages the patient to share their true feelings, avoiding assumptions on your part. Empathy can also be expressed nonverbally, such as by placing a hand on the patient's arm or offering tissues to a crying patient. Demonstrating your concern helps uncover vital aspects of the patient's experience.

Once the patient has opened up about their feelings, respond with understanding and acceptance. Your replies can be simple, yet profound, statements like, *"I can't imagine how difficult this must be for you,"* *"That sounds very upsetting,"* or *"You must be feeling very sad."* For a response to truly be empathic, it must resonate with what the patient is feeling, conveying that you are genuinely sharing in their emotional experience.

Summarization

Summarizing the patient's story during the interview fulfills multiple objectives. It demonstrates that you have been listening attentively and clarifies what you understand and what you still need to learn. It allows for the correction of any misunderstandings and organizes the clinical reasoning process, making the relationship more collaborative.

For example, you might say, *"Let me ensure I've got everything. You've mentioned a cough for the last 3 days, worsening at night, accompanied by yellow phlegm. You've experienced no fever or shortness of breath, but you've noted congestion and difficulty breathing through your nose."* Encouraging further input with a thoughtful pause or a prompt like, *"Anything else?"* invites the patient to provide additional details or correct any inaccuracies.

Summarizing at various stages of the interview helps structure the session, particularly during transitions. It aids in organizing your clinical thoughts and sharing your diagnostic process with the patient, fostering a more collaborative relationship. Additionally, this strategy is beneficial for guiding clinicians, especially when they are uncertain about the next question to ask.

Transitions

Signposting or clearly stating when the interview is moving to a new topic or phase helps prepare the patient and reduces anxiety, facilitating a smoother interaction.

As you transition from taking the patient's history to conducting the physical examination, use concise transitional phrases to guide the patient, such as, *"I will now ask you some questions about your previous health."* Clearly communicate the next steps to ensure the patient knows what to expect. For example, you might say, *"Before we proceed to review your medications, is there anything else you'd like to share about your past health issues?"* When moving to the physical examination, inform the patient politely, *"We will now begin the physical examination. I'll leave the room for a moment. Please change into this gown."* This approach helps maintain a smooth and respectful flow of the consultation, ensuring the patient feels informed and comfortable with the process.

Validation

Validating the patient's emotional experience, especially in the context of distressing events or diagnoses, affirms their feelings as legitimate and understandable, enhancing trust and rapport.

A patient involved in a car accident, even without physical injuries, may experience significant distress. Acknowledging their feelings with a statement such as, *"It must have been frightening to be in that accident. Car accidents can be deeply unsettling, highlighting our vulnerability. It's completely natural that you're feeling upset,"* validates their emotions as both legitimate and understandable.

Partnering

Demonstrating a commitment to a lasting relationship offers patients the reassurance of ongoing care and support, an aspect that is especially comforting in hospital environments or throughout long-term treatment. This assurance of support is critical, particularly for students working in hospital settings, as it significantly impacts patient comfort and trust.

For instance, saying, *“I’m here for you, and we’ll navigate this together,”* or *“We’re committed to providing you continuous care and support at every step,”* can profoundly convey your dedication to their well-being and foster a strong sense of partnership.

Empowering the Patient

The relationship between clinician and patient is fundamentally unequal. As a student, you might initially feel inexperienced, but this feeling evolves as you gain more clinical expertise. On the other hand, patients often encounter vulnerabilities due to pain, anxiety about symptoms, or the overwhelming nature of accessing health care. Factors such as gender, ethnicity, race, and socioeconomic status further intensify this imbalance of power. Nevertheless, recognize that patients bear the ultimate responsibility for their own health care.¹⁴ Empowering patients by encouraging them to ask questions, voice their concerns, and critically evaluate your recommendations significantly increases the likelihood that they will follow your advice, adopt healthier lifestyles, or adhere to prescribed medication regimens.¹²

Box 2-2 outlines strategies for equalizing the power dynamic with your patients. While many of these techniques have been addressed, emphasizing the importance of patients taking charge of their health is a key principle worth reiterating.

Reassurance

When patients are anxious or upset, offering quick reassurance like *“Don’t worry. Everything is going to be alright”* might seem helpful, but for clinicians, such assurances can be premature and counterproductive. In some cases, they might even mislead and hinder further communication. Patients could interpret these reassurances as a sign of the clinician’s discomfort with anxiety or an underestimation of their distress.

The first step to effective reassurance is simply identifying and acknowledging the patient’s feelings. Saying something like, *“You seem upset today,”* can foster a connection and show empathy. Genuine reassurance should follow the completion of the interview, physical examination, and any necessary lab tests. At this juncture, explaining your findings and addressing concerns directly is more impactful. Reassurance becomes meaningful and appropriate when the patient feels their issues are understood and actively managed.

Box 2-2. Empowering the Patient: Techniques for Sharing Power

- Evoke the patient’s perspective.
- Convey interest in the person, not just the problem.
- Follow the patient’s leads.
- Elicit and validate emotional content.
- Share information with the patient, especially at transition points during the visit.
- Make your clinical reasoning transparent to the patient.
- Reveal the limits of your knowledge.

ADDRESSING SENSITIVE TOPICS

In the upcoming chapters, you will discover how clinicians navigate discussions on a range of sensitive issues. Engaging in conversations about topics like alcohol or drug use, sexual practices, death, financial worries, racial and ethnic biases, domestic violence, mental health, physical abnormalities, and bowel function can be challenging, especially for those new to the field or when dealing with unfamiliar patients. Such discussions are often hindered by societal norms, even for experienced clinicians, as these subjects may provoke strong reactions stemming from personal, cultural, and societal beliefs.

To effectively address these sensitive subjects, several strategies can enhance your comfort and proficiency (Box 2-3). We recommend engaging with both clinical and general literature on these topics, discussing your concerns with peers and mentors, participating in courses designed to explore your emotional responses, and reflecting on your own experiences. Maximizing these resources is crucial. Observing seasoned clinicians handle these conversations and practicing their techniques can also be invaluable. As you gain experience, you will find your ability to discuss these topics with ease and sensitivity improves over time.

Box 2-3. Guidelines for Broaching Sensitive Topics ¹⁵

- **Embrace nonjudgmental engagement:** The cornerstone principle is maintaining a nonjudgmental stance. Your objective is to learn from your patient and facilitate their journey toward improved health. Demonstrating acceptance is paramount to achieving this aim.
- **Clarify the purpose of your inquiries:** To mitigate patient anxiety, communicate the rationale behind your questions. For instance, you might tell a patient, *“In order to provide you with the best possible care, I need to ask some questions regarding your sexual health and practices.”*
- **Identify and use appropriate opening questions:** Begin discussions on sensitive topics with carefully chosen questions, and understand the specific information required for a comprehensive assessment and collaborative planning.
- **Acknowledge your own discomfort:** Recognize and admit any personal discomfort you may feel about the topic. Ignoring these feelings can lead to avoidance of the subject, which is counterproductive.

van de Poel K, Vanagt E, Schrimpf U, Gasiorek J. *Communication Skills for Foreign and Mobile Medical Professionals*. Springer; 2013.

PRACTICING LEGAL AND ETHICAL COMMUNICATION

Informed Consent

A patient’s consent to a procedure or treatment is more than simply signing a form. *Informed consent* involves a comprehensive communication process where a clinician thoroughly educates a patient about the risks, benefits, and alternatives associated with a procedure or intervention. This educational dialogue is crucial for ensuring that patients make informed decisions regarding their

health care.¹⁶ Box 2-4 describes the required elements for documentation of the informed consent discussion.

Clinicians bear both a legal and ethical obligation to adhere to this consent process meticulously, ensuring that no critical details are omitted.

Recognizing that each patient's situation is unique is key to obtaining informed consent. Variables such as personal circumstances, cultural background, and cognitive abilities can significantly influence decision-making capabilities. It is imperative to confirm that the patient has the *decisional capacity*. If doubts arise, consultation with the designated health care proxy becomes necessary. Effective communication involves using simple, nonpatronizing language and avoiding medical jargon. Using the teach-back method can be invaluable in gauging the patient's comprehension of the information.

See Chapter 1 , Foundational Skills Essential to the Clinical Encounter for further discussion of the teach-back method, p. 10.

Providing additional resources for the patient to explore independently, like brochures, websites, or videos, can further aid in their understanding. Engaging in an open dialogue by encouraging questions and being available for follow-up discussions is fundamental.

Every patient with decisional capacity retains the right to either consent to or decline procedures or treatments once fully informed.

See determination of decisional capacity in Chapter 1 , Foundational Skills Essential to the Clinical Encounter, p. 16.

Box 2-4. Essential Elements for Informed Consent Documentation

Element	Description	Sample Statements
Nature of the procedure or treatment	Clear explanation of what the patient should expect	<i>"This procedure involves..." "It's designed to..." "You can expect..."</i>
Risks and benefits	Honest discussion about the potential outcomes and the likelihood of success or complications	<i>"While many patients experience improvement, there are risks such as..." "The benefits include..."</i>
Reasonable alternatives	Information on other viable options, including opting for no treatment	<i>"Apart from this procedure, other options include ... including the option of not undergoing any treatment at this time."</i>
Risks and benefits of alternatives	Comparison to help the patient understand their choices fully	<i>"Comparing this to the alternatives, the risks are... However, the benefits might be..."</i>
Assessment of patient understanding	Verification that the patient grasps the implications of their decisions based on the information provided	<i>"Do you feel you understand the differences between...?" "What are your thoughts on the risks and benefits I discussed?"</i>

Collaborating with a Medical Interpreter

A few words in the language that is most comfortable for your patient may enhance rapport, but they are no substitute for the full story. Even fluent speakers might miss subtle meanings in specific words. Likewise, relying on family members for translation can breach confidentiality and result in distorted or incomplete information. Key details may be lost as lengthy explanations are condensed. The best solution is a “*cultural navigator*”: a neutral interpreter skilled in both the relevant languages and cultures. Nonetheless, even professionals might not fully understand the nuances of all subcultures.^{17, 18}

When working with an interpreter, start by building a connection and outlining the key information needed (Box 2-5). Ask that the interpreter translate everything without summarizing. To aid clarity, pose questions that are concise and direct. Clearly define your objectives for each part of the patient’s history. Before starting, arrange the seating to maintain direct eye contact with the patient, which fosters a more personal connection. Speak directly to the patient, using “you” instead of referring to them in the third person, and position the interpreter to minimize the need to turn your head.

With linguistic diversity on the rise, clinicians increasingly rely on interpretation technologies to bridge the gap between languages and cultures. Box 2-6 provides a comparative overview of the current technologic solutions available for medical interpretation, highlighting their distinctive features and optimal use cases.

USING ADVANCED COMMUNICATION STRATEGIES

Appropriate Verbal Communication

As clinicians, we must be careful in *what* we say as well as *how* we say it. The effectiveness of the clinical encounter rests on the use of appropriate language. This can also enhance patient rapport and lead to a satisfying patient–clinician relationship.

Use Understandable Language. Using simple and clear language is key when talking to patients, no matter how much they know about health. Use easy-to-understand words (avoiding medical terms and abbreviations) and short sentences, and stick to the most important information.

For example, instead of, “*Does the pain radiate?*” ask, “*Does the pain move anywhere?*” If you use a word that is not understood, explain it in a way that is easier to understand. Also, choose specific and clear words over vague ones like “*a little bit,*” “*common,*” or “*rare.*” Always communicate clearly with all patients, no matter their education, wealth, or cultural background.

Box 2-5. Guidelines for Working with an Interpreter: “INTERPRET”

I	Introductions: Make sure to introduce all the individuals in the room. During the introduction, include what role each individual will play.
N	Note goals: Note the goals of the interview. What is the diagnosis? What will the treatment entail? Will there be any follow-up?
T	Transparency: Let the patient know that everything said will be interpreted throughout the session.
E	Ethics: Use qualified interpreters (not family members or children) when conducting an interview. Qualified interpreters allow the patient to maintain autonomy and make informed decisions about their care.
R	Respect beliefs: Patients with limited English proficiency (LEP) may have cultural beliefs that need to be considered as well. The interpreter may be able to serve as a cultural broker and help explain any cultural beliefs that may exist.
P	Patient focus: The patient should remain the focus of the encounter. Providers should interact with the patient and not the interpreter. Make sure to ask and address any questions the patient may have before ending the encounter. If you do not have trained interpreters on staff, the patient may not be able to call in with questions.
R	Retain control: As the provider, you must remain in control of the interaction and not let the patient or the interpreter take over the conversation.
E	Explain: Use simple language and short sentences when working with an interpreter. This will ensure that comparable words can be found in the second language and that all the information can be conveyed clearly.
T	Thanks: Thank the interpreter and the patient for their time. On the chart, note that the patient needs an interpreter and who served as interpreter.

Source: Administration for Children and Families. U.S. Department of Health and Human Services. INTERPRET tool: working with interpreters in cultural settings. Accessed March 17, 2024. https://www.acf.hhs.gov/sites/default/files/documents/otip/hhs_clas_interpret_tool.pdf

Remember, even with clear language, patients can feel overwhelmed if they get too much information at once. During visits, focus on one to three main points and repeat them often. A good way to make sure patients understand and engage in their care is by using the “Ask Me Three” method. This encourages patients to ask three important questions in each visit.²⁰

1. What is my main problem?
2. What do I need to do?
3. Why is it important for me to do this?

Modifying this approach to “Tell Them Three” can also help clinicians keep their message focused and simple. Another approach to make sure your patient understands you is the teach-back method.²¹ ,²² Keep in mind that “teach back” is not a test of the patient’s knowledge but a test of how well you explained things in a manner your patient understands.

See Chapter 1 , Foundational Skills Essential to the Clinical Encounter for further discussion of the teach-back method, p. 10.

Use Nonstigmatizing Language. It is important to use language that respects and affirms the individual’s dignity during clinical interactions. Sometimes, we might inadvertently use phrases or terms in the clinical setting that could feel dehumanizing to the patient, perpetuating stigma and marginalization instead of fostering a supportive environment.²³ The language we choose should mirror the full spectrum of human identity, recognizing each person’s potential for change and growth. Words that carry unintended stigma can alienate patients, deter them from seeking necessary support or treatment, and reinforce harmful stereotypes.²⁴

For instance, instead of asking, “*Do you still consider yourself a drug addict?*” or “*Are you wheelchair bound?*” consider phrasing these questions as “*Do you still see yourself as someone who has struggled with substance use?*” or “*Do you use a wheelchair daily?*”

Box 2-6. Comparative Overview of Medical Interpretation Technologies